

7.35.3 NMAC: proposed amendments to the twenty-four sections outside the practicum

Working draft v2, July 26, 2026. Rule hearing August 28, 2026. Not a filing, not submitted, and not adopted rule text.

- This document covers the twenty-four sections of 7.35.3 NMAC that the practicum amendment draft does not reach: 7.35.3.1 through .13, .15, .16, .17, and .21 through .28. It is in four parts, with five addenda at the end.
- Nothing here reopens the practicum draft, which covers 7.35.3.14, .18, .19, Paragraph (5) of Subsection H of .20, and a proposed .29. Read against working draft v9 of that document. Its *amendments/SOURCE-OF-TRUTH.md* assigns to this document each item drafted or recorded here, and Addendum B records every dependency in both directions.
- Twenty-four provisions are amended.** Every change is a mechanical correction, a conforming amendment between two provisions of the same rule, or a restoration of wording the July 9, 2026 draft carried. No change introduces a figure or a policy choice that no source supplies.
- Twenty-one further defects are recorded and not drafted.** Each carries a note at its provision stating the issue, the choices, and who decides, and all twenty-one are listed with the reason in Addendum C. The commonest reason is that the fix requires a figure no source carries.
- The two BLOCKING findings in this scope are both reached. The third-party evaluator conflict is drafted at 7.35.3.16 A and again at 7.35.3.16 C(2) (b), so either or both may be adopted. The registration on which 7.35.3.14 C conditions healing center staff authority is drafted at 7.35.3.11 A(11), which is where v9 records that the fix belongs.

The columns. Left is the rule as published, verbatim. Right is the proposed amendment: underlined green inserted, ~~struck red deleted~~, unmarked text carried forward unchanged. 60 from 7.35.3.15 B marks a figure taken from another provision of the same rule rather than newly drafted. A provision shown with no amendment is reproduced because it carries a finding. The source note and, where there is one, the review note run the full width beneath both columns.

Contents

PART	SECTIONS	WHAT IT COVERS
I. Framework and defined terms	.2, .3, .5, .7	7.35.3.1 through 7.35.3.7. The dependency of this part on the definitions in 7.35.2.7 NMAC, and the four definitions a single provision of the published rule supplies.
II. Educational programs	.10, .12, .15, .16, .17	7.35.3.10, .12, .15, .16 and .17. The third-party evaluator conflict, the out-of-jurisdiction pathway, and the reporting and consultation obligations.
III. Locations and oversight	.11, .20, .21	7.35.3.11, .20 and .21. Healing center and other approved location applications, the registration on which healing center staff authority depends, and department assessments.
IV. Patients, applicants and process	.8, .9, .13, .23, .24, .25, .27	7.35.3.8, .9, .13 and .22 through .28. Enrollment and certification applications, complaints, review of denials, and discipline.
Addendum A	All 28, and 7.35.2 NMAC	Terms this part uses and no part defines, counted section by section in both parts.
Addendum B	Both scopes	Dependencies between these sections and the provisions the practicum draft reaches.
Addendum C	These 24	Every defect recorded and not drafted, with the reason nothing is proposed.
Addendum D	All 28	Numbering, dates and drafting form across the whole part.
Addendum E	Every review note	The questions this document leaves open, one line each, grouped by who decides.

The defined terms are the largest finding. 7.35.3.7 NMAC provides in full: "The definitions in 7.35.2.7 NMAC apply to this part." Sixteen terms that carry regulatory consequence in this part are defined in neither part. Six are drafted at 7.35.3.7, because a single provision of the published rule supplies the content.

Ten are not, and the two largest are healing center, 54 occurrences, and certifying clinician, 53 occurrences. Addendum A counts every one of them, section by section, in both parts, and assembles for each of the ten the provisions of 7.35.3 NMAC, 7.35.2 NMAC and the Medical Psilocybin Act that bear on it, so a definition can be decided from assembled material rather than composed from scratch.

Part I. Framework and defined terms

7.35.3.1 through 7.35.3.7. The dependency of this part on the definitions in 7.35.2.7 NMAC, and the four definitions a single provision of the published rule supplies.

7.35.3.2 Scope

PART I

CURRENT LANGUAGE	PROPOSED AMENDMENT
Proposed rule as published July 23, 2026	Draft for review. Not filed, not adopted
Entire section, scope 7.35.3.2 SCOPE: This rule applies to all patients, practitioners, clinicians, facilitators, healing centers and other approved locations, and educational programs who participate or seek to participate in the New Mexico medical psilocybin program.	Entire section, scope Not amended. This provision is reproduced because it carries a finding recorded below.
SOURCE Not amended. Recorded for the terminology it fixes. 7.35.3.2 NMAC, page 1, lists the regulated classes as "patients, practitioners, clinicians, facilitators, healing centers and other approved locations, and educational programs". Of those, 7.35.2.7 NMAC defines practitioner, clinician and approved location, and does not define facilitator, healing center or educational program. Carried over from the July 9, 2026 draft.	
Please review. The scope names six regulated classes, and 7.35.2.7 NMAC defines three of them. The rule uses "clinician" here and "certifying clinician" 53 times elsewhere, and 7.35.2.7 NMAC defines the first and not the second. Two choices. Leave the scope as published and rely on the definitions drafted at 7.35.3.7 NMAC to carry the undefined terms, or conform this section to the vocabulary the rest of the part uses. The Training and Education Committee and Department of Health staff decide which, and the choice governs the definition slate at 7.35.3.7 NMAC.	

7.35.3.3 Statutory authority

PART I

CURRENT LANGUAGE	PROPOSED AMENDMENT
Proposed rule as published July 23, 2026	Draft for review. Not filed, not adopted
Entire section, statutory authority 7.35.3.3 STATUTORY AUTHORITY: This rule is promulgated pursuant to the following statutory authorities: the New Mexico Department of Health Act, Subsection E of Section 9-7-6 NMSA 1978; and the Medical Psilocybin Act, Section 26-2D-7, NMSA 1978.	Entire section, statutory authority <i>Not amended. This provision is reproduced because it carries a finding recorded below.</i>
SOURCE Not amended. Recorded as a fact. 7.35.3.3 NMAC, page 1, cites "the Medical Psilocybin Act, Section 26-2D-7, NMSA 1978". Section 1 of the Act provides that Sections 1 through 11 of the act may be cited as the Medical Psilocybin Act. Section 7 of the Act is the section creating the program in the department and directing what the department shall establish. This record does not verify the codification. Carried over from the July 9, 2026 draft.	
Please review. This document does not verify any NMSA 1978 section number, and the codification asserted here has not been checked against the enacted text in this record. Two choices. Leave the citation as published, or ask the department to confirm the codification before the rule is filed. Department of Health staff and department counsel decide, and no board action is needed either way.	

7.35.3.5 Effective date

PART I

CURRENT LANGUAGE

Proposed rule as published July 23, 2026

PROPOSED AMENDMENT

Draft for review. Not filed, not adopted

Entire section, effective date

7.35.3.5 EFFECTIVE DATE: xx/xx, 2026, unless a later date is cited at the end of a section.

Entire section, effective date*Not amended. This provision is reproduced because it carries a finding recorded below.*

SOURCE Not amended. Recorded for its interaction with the history notes. 7.35.3.5 NMAC, page 1, reads “xx/xx, 2026, unless a later date is cited at the end of a section”. Twenty-six of the twenty-eight history notes in the rule read “xx/xx/2026”. The notes for 7.35.3.27 and 7.35.3.28, page 19, read “9/22/2026”. Carried over from the July 9, 2026 draft, which read “XXXX,XX,2026”.

Please review. Because 7.35.3.5 NMAC gives effect to a later date cited at the end of a section, the rule as published takes effect on an unfilled date except for 7.35.3.27 and 7.35.3.28, which take effect September 22, 2026. The rule hearing is set for August 28, 2026. No source in this record supplies the intended effective date, so none is drafted. Three choices. Conform the two history notes to the placeholder the other twenty-six carry, fill 7.35.3.5 NMAC with a date and leave the two notes as they stand if a later date for those two sections is intended, or fill all twenty-eight with one date. Department of Health staff decide, because the department sets the filing and effective dates.

7.35.3.7 Definitions

PART I

CURRENT LANGUAGE

Proposed rule as published July 23, 2026

PROPOSED AMENDMENT

Draft for review. Not filed, not adopted

Entire section, definitions

7.35.3.7 DEFINITIONS: The definitions in 7.35.2.7 NMAC apply to this part.

Entire section, definitions7.35.3.7 DEFINITIONS: A. The definitions in 7.35.2.7 NMAC apply to this part.**B. In addition to the definitions in 7.35.2.7 NMAC, the following definitions apply to this part:****(1) “Certificant” means a practitioner, certifying clinician, facilitator, healing center or other approved location, or educational program that holds a certification issued by the department under this part.****(2) “Facilitator” means an individual who is certified by the department under this part to work alongside a practitioner during medical psilocybin services, under the direct supervision of a practitioner, and to provide peer support to qualified patients as well as logistical and administrative support to the practitioner and to the healing center.****(3) “Other approved location” means a physical location at which medical psilocybin is intended to be consumed, that is not the location of a healing center, and that holds a temporary certification issued by the department under Subsection B of 7.35.3.11 NMAC.****(4) “Practicum” means the supervised practice training required of an individual who seeks to become certified as a practitioner or facilitator under 7.35.3.19 NMAC.****(5) “Registrant of another approved location” means the practitioner or facilitator who holds a certification of an other approved location issued under Subsection B of 7.35.3.11 NMAC.****(6) “Student” means an individual who is registered with a psilocybin educational program certified by the department under this part.**

SOURCE Six definitions are added. Each is drawn from a single provision of the rule as published, cited here, and none is composed for this draft. Certificant: 7.35.3.26 NMAC, page 16, which names the six classes that hold a certification. Facilitator: 7.35.3.13 NMAC Subsection B, page 8, which states the scope of work, together with 7.35.3.9 NMAC Subsection F, page 3, which requires certification. Other approved location: 7.35.3.11 NMAC Subsection B, page 6, which provides that such locations “are physical locations at which medical psilocybin is intended to be consumed, that are not locations of healing centers” and that they hold a temporary certification. Practicum: 7.35.3.19 NMAC Subsection A, page 12, which provides that an individual who seeks to become certified as a practitioner or facilitator “shall participate in supervised practice training” and gives that training the name practicum; that section belongs to the practicum amendment draft, whose working draft v9 amends the hour figures in the same subsection and keeps the quoted phrase, and Addendum B records the dependency. Registrant of another approved location: 7.35.3.11 NMAC Subsection B, page 6. Student: 7.35.3.20 NMAC Paragraph (5) of Subsection H, page 14. Section 7.35.3.7 as published reads in full: “The definitions in 7.35.2.7 NMAC apply to this part.” This section is new; the July 9, 2026 draft carried no definitions section. Addendum A maps every undefined term and assembles, for each term left undrafted, the provisions that bear on it. Amending 7.35.2 NMAC is a separate rulemaking, because that part was adopted effective June 23, 2026, so any term this part needs and 7.35.2.7 NMAC does not supply has to be carried here.

Please review. Two questions here, and the second is the larger one. First, the slate. Sixteen terms that carry regulatory consequence in this part are defined nowhere: the six definitions drafted here are the only ones a single provision of the rule supplies, and ten remain, including healing center, which appears 54 times, and certifying clinician, which appears 53 times. No source in this record defines either, so neither is drafted. Addendum A lists all sixteen with counts and assembles, for each of the ten, every provision of 7.35.3 NMAC, 7.35.2 NMAC and the Medical Psilocybin Act that bears on the term. The Training and Education Committee decides which to send to the department, and the department decides what each says. Second, the facilitator. Section 3(B) of the Medical Psilocybin Act defines a clinician as an approved health care provider licensed in New Mexico who holds a permit from the department to provide medical services to qualified patients. Section 5 of the Act exempts from arrest, prosecution and penalty a producer, a clinician and a qualified patient, and Paragraph (2) of Subsection B of Section 5 makes lawful the conduct of “a clinician administering or a qualified patient taking psilocybin in an approved setting”. A facilitator holds no professional license under this rule: Subsection F of 7.35.3.9 NMAC, page 3, requires no license of a facilitator applicant. Subsection B of 7.35.3.14 NMAC, page 9, nonetheless authorizes facilitators to possess medical psilocybin products and to provide them to qualified patients. Whether a facilitator falls within Section 3(B) of the Act, and so within the Section 5 exemption, is a question of statutory construction that this record cannot settle and that a definition in this part cannot cure if the answer is no. Two choices once it is answered: if a facilitator is within Section 3(B), nothing needs to change; if not, the authority in Subsection B of 7.35.3.14 NMAC has to be narrowed or the Act amended. Department of Health staff and department counsel decide it, and the board should be told the answer before the rule is filed. Two things bear on this and neither is in doubt. The practicum amendment draft reads the same Paragraph (2) of Subsection B of Section 5 against the same Subsection H of Section 3, which defines a qualified patient as “a patient whose clinician has judged the patient to be a medically appropriate candidate for the use of medical psilocybin based on being diagnosed with a qualifying condition”, and concludes that a participant who is not a qualified patient falls outside the provision; its proposed 7.35.3.29 accordingly states the statutory amendment it would need and conditions the whole section on that amendment. The question recorded here is the other half of the same provision, about who may administer rather than who may take. Separately, the position of a student is not in doubt and is not raised here: the practicum draft locates the provision of the medicine in the practitioner or the healing center under Subsections A and C of 7.35.3.14 NMAC, and 7.35.3.20 NMAC Subsection D, page 14, provides that “students completing their practicums may be present during an administration session”.

Part II. Educational programs

7.35.3.10, .12, .15, .16 and .17. The third-party evaluator conflict, the out-of-jurisdiction pathway, and the reporting and consultation obligations.

7.35.3.10 Application process based on educational programs from other jurisdictions

PART II

CURRENT LANGUAGE

Proposed rule as published July 23, 2026

PROPOSED AMENDMENT

Draft for review. Not filed, not adopted

Paragraph (2) of Subsection A, additions to the list

(2) Educational programs shall be added to the list when an application for certification as a certifying clinician, practitioner, or facilitator is submitted and approved based on the applicant's completion of the out-of-state educational program.

Paragraph (2) of Subsection A, additions to the list

Not amended. This provision is reproduced because it carries a finding recorded below.

SOURCE Not amended. Recorded for the sequence it creates. 7.35.3.10 NMAC Paragraph (2) of Subsection A, page 4, is the only route by which a program reaches the list. Carried over from the July 9, 2026 draft, where the section was titled "RECIPROCITY FOR CERTIFYING CLINICIANS, PRACTITIONERS AND FACILITATORS".

Please review. A program reaches the list only when an individual application relying on that program is approved, and an application filed on or after January 1, 2028 needs either a third-party evaluation or a program already on the list. On the first day the list is empty, so the first applicants must supply an evaluation, and finding M6 of `analysis/july23-rule-concerns.md` records that the list can only populate after the fact. Two choices. Leave the paragraph as published and rely on the amendment drafted at Paragraph (2) of Subsection B of this section, which lets an applicant point to the list once a program is on it, or add a route by which the department may place a program on the list on its own initiative. The second is a new grant of authority and no source in this record proposes its terms, so it is not drafted. The Training and Education Committee decides whether to ask for it, and Department of Health staff decide its terms.

Paragraph (2) of Subsection B, documentation of equivalency

(2) Documentation of either: (a) Completion of a government approved psilocybin educational program from another jurisdiction, which shall include: (i) Documentation that the curriculum of the educational program is equivalent to the didactic educational requirements applicable for New Mexico certification; and (ii) For all applications received on or after January 1, 2028, documentation of a third-party evaluation of the curriculum demonstrating such equivalency; (b) Completion of a psilocybin educational program from another jurisdiction that is developed in collaboration with higher education institutions or government-approved psychedelic research studies, which shall include: (i) Documentation that the curriculum of the educational program is equivalent to the didactic educational requirements applicable for New Mexico certification, and (ii) For all applications received on or after January 1, 2028, documentation of a third-party evaluation of the curriculum demonstrating such equivalency; or (c) Completion of an educational program that is currently included on the list of department-approved psilocybin educational programs from other jurisdictions.

Paragraph (2) of Subsection B, documentation of equivalency

(2) Documentation of either: (a) Completion of a government approved psilocybin educational program from another jurisdiction, which shall include: (i) Documentation that the curriculum of the educational program is equivalent to the didactic educational requirements applicable for New Mexico certification; and (ii) For all applications received on or after January 1, 2028, documentation of a third-party evaluation of the curriculum demonstrating such equivalency, except that an applicant may satisfy this item by documenting that the educational program is included on the list maintained under Subsection A of this section; (b) Completion of a psilocybin educational program from another jurisdiction that is developed in collaboration with higher education institutions or government-approved psychedelic research studies, which shall include: (i) Documentation that the curriculum of the educational program is equivalent to the didactic educational requirements applicable for New Mexico certification, and (ii) For all applications received on or after January 1, 2028, documentation of a third-party evaluation of the curriculum demonstrating such equivalency, except that an applicant may satisfy this item by documenting that the educational program is included on the list maintained under Subsection A of this section; or (c) Completion of an educational program that is currently included on the list of department-approved psilocybin educational programs from other jurisdictions.

SOURCE Conforming amendment. Items (a)(ii) and (b)(ii) of 7.35.3.10 NMAC Paragraph (2) of Subsection B, page 4, require an individual applicant to produce a third-party evaluation of a curriculum. Under 7.35.3.16 NMAC Subsection A, page 10, a third-party evaluation is commissioned by a psilocybin educational program, not by an individual. Item (c) of the same paragraph already lets an applicant rely on the list maintained under Subsection A, and this amendment extends that route to items (a)(ii) and (b)(ii). No figure is added.

Subsection C, re-application after denial

C. Application approval and denial; certification period: If the application is approved, the certification shall be effective on the date of notice and shall be valid for two years. If the department denies an application, the department shall provide notice of the denial in accordance with this rule. If the application is denied, the applicant may re-apply within six months. An applicant who is denied a second time may not re-apply for six months from the denial. An applicant whose application is denied may appeal the denial in accordance with this rule.

Subsection C, re-application after denial

C. Application approval and denial; certification period: If the application is approved, the certification shall be effective on the date of notice and shall be valid for two years. If the department denies an application, the department shall provide notice of the denial in accordance with this rule. If the application is denied, the applicant may re-apply ~~within~~ **no earlier than** six months **after the date of the denial**. An applicant who is denied a second time may not re-apply for six months from the denial. An applicant whose application is denied may appeal the denial in accordance with this rule.

SOURCE Conforming amendment, one of four in the rule. 7.35.3.10 NMAC Subsection C, page 4, reads "the applicant may re-apply within six months", which on its face sets a deadline. The next sentence of the same subsection reads "An applicant who is denied a second time may not re-apply for six months from the denial", which sets a waiting period. The amendment conforms the first sentence to the second. The same pair appears at 7.35.3.9 NMAC Paragraphs (5) and (6) of Subsection A, page 2; 7.35.3.11 NMAC Subsection D, page 7; and 7.35.3.12 NMAC Subsection D, page 8. All four are drafted the same way in this document. No figure is added.

Please review. The amendment reads the six months as a waiting period, because the following sentence of the same subsection sets a waiting period for a second denial and the two sentences otherwise contradict each other. The other reading is that a denied applicant has six months in which to re-apply and loses the right afterwards. Finding M21 of `analysis/july23-rule-concerns.md` records the ambiguity. Department of Health staff decide which reading was intended, and the same answer governs all four places the sentence appears.

Paragraph (1) of Subsection D, waiver of practicum requirements

D. Waiver of practicum requirements: (1) An applicant for certification as a practitioner or facilitator on the basis of an educational program from another jurisdiction, whose application is received by the department by December 31, 2027, shall not be required to complete the full practicum hours required by New Mexico, but shall demonstrate completion of at least 40 hours of contact time with a minimum of two separate individual sessions and two separate group sessions for preparation, administration, and integrative therapy with psilocybin treatment.

Paragraph (1) of Subsection D, waiver of practicum requirements

Not amended. This provision is reproduced because it carries a finding recorded below.

SOURCE Not amended. Reproduced for the conflict it makes with Paragraph (4) of Subsection G of 7.35.3.19 NMAC, page 13, which this document does not reach. For the same 40 hours of contact time, this paragraph, at page 5, requires “two separate group sessions” and Paragraph (4) of Subsection G of 7.35.3.19 NMAC requires “A minimum of one group session”. Both descend from the July 9, 2026 draft, which carried the same waiver twice and read “a minimum of two separate individual sessions or two separate group sessions” in both places. The published rule departed from that wording differently in each place, so no restoration of the July 9 wording can conform the two provisions. The practicum amendment document records the same conflict at its re-lettered 7.35.3.19 K and amends neither provision. Addendum B and Addendum C carry the cross-references.

Please review. Two waivers of the practicum hours requirement each set a 40-hour floor for applications received by December 31, 2027, and they disagree on the group sessions inside those hours. This paragraph requires “two separate group sessions”; Paragraph (4) of Subsection G of 7.35.3.19 NMAC, page 13, accepts “A minimum of one group session”. This paragraph is the stricter reading, and it is the one that names an applicant relying on an educational program from another jurisdiction. The two scopes overlap: Paragraph (3) of Subsection G of 7.35.3.19 NMAC extends that section's waiver to a graduate of a program “that the department has included on the department-approved list of educational programs by December 31, 2027”, and the list maintained under Subsection A of this section is a list of programs from other jurisdictions, so an out-of-jurisdiction applicant whose program is on the list appears to qualify under both waivers at once. Such an applicant whose 40 documented hours include one group session satisfies Subsection G of 7.35.3.19 NMAC and fails this paragraph, and the rule does not say which governs. Three choices. Conform this paragraph to the one group session; conform Paragraph (4) of Subsection G of 7.35.3.19 NMAC to the two group sessions here, which is in the practicum amendment document's scope and would have to be drafted there; or state which waiver an applicant relying on an out-of-jurisdiction program proceeds under. Each is a choice between two figures already in the rule, and the July 9, 2026 draft cannot break the tie, because it read “or two separate group sessions” in both places, a third reading the published rule dropped. The Training and Education Committee decides which to send forward, and the answer has to be coordinated with the practicum amendment document, which records the same conflict and amends neither provision.

7.35.3.12 Application process for psilocybin educational programs

PART II

CURRENT LANGUAGE

Proposed rule as published July 23, 2026

Subsection D, re-application after denial

D. Approval and denial; certification period: If the application is approved, the certification shall be effective on the date of notice and shall be valid for two years. If the department denies an application, the department shall provide notice of the denial in accordance with this rule. If the application is denied, the applicant may re-apply within six months. An applicant who is denied a second time may not re-apply for six months from the denial. An applicant whose application is denied may appeal the denial in accordance with this rule.

PROPOSED AMENDMENT

Draft for review. Not filed, not adopted

Subsection D, re-application after denial

D. Approval and denial; certification period: If the application is approved, the certification shall be effective on the date of notice and shall be valid for two years. If the department denies an application, the department shall provide notice of the denial in accordance with this rule. If the application is denied, the applicant may re-apply ~~within~~ **no earlier than** six months **after the date of the denial**. An applicant who is denied a second time may not re-apply for six months from the denial. An applicant whose application is denied may appeal the denial in accordance with this rule.

SOURCE Conforming amendment. Same defect and same fix as 7.35.3.10 NMAC Subsection C. 7.35.3.12 NMAC Subsection D, page 8. Carried over from the July 9, 2026 draft.

7.35.3.15 Psilocybin educational programs; required reporting and curriculum approval

PART II

CURRENT LANGUAGE

Proposed rule as published July 23, 2026

PROPOSED AMENDMENT

Draft for review. Not filed, not adopted

Subsection A, required reporting of updates

A. Required reporting of updates: A department-certified psilocybin educational program shall notify the department, through the department-approved electronic system designated by the department, of any of the following changes, and shall not implement the change until written approval is issued by the department: (1) Any revision to the program's curriculum; revised curricula shall be submitted to the department for review and may not be implemented until the department grants approval; (2) Any change in ownership, including changes in controlling interest or organizational structure; (3) Any change in physical locations utilized for instruction or educational activities; (4) Any modification, termination, or addition of practicum site agreements; and (5) Any change in instructional staff, including submission of curricula vitae or résumés for all newly added instructors.

Subsection A, required reporting of updates

A. Required reporting of updates: A department-certified psilocybin educational program shall notify the department, through the department-approved electronic system designated by the department, of any of the following changes, and shall not implement the change until written approval is issued by the department. The department shall notify the program of its approval or denial of a change reported under this subsection within 60 calendar days of receiving the report 60 from 7.35.3.15 B : (1) Any revision to the program's curriculum; revised curricula shall be submitted to the department for review and may not be implemented until the department grants approval; (2) Any change in ownership, including changes in controlling interest or organizational structure; (3) Any change in physical locations utilized for instruction or educational activities; (4) Any modification, termination, or addition of practicum site agreements; and (5) Any change in instructional staff, including submission of curricula vitae or résumés for all newly added instructors.

SOURCE The 60 calendar days are taken from 7.35.3.15 NMAC Subsection B, page 9, which gives the department 60 calendar days to decide a curriculum approval application. No new figure is introduced. Subsection A as published bars a program from implementing a reported change "until written approval is issued by the department" and sets no period within which the department must act. Carried over from the July 9, 2026 draft.

Please review. Finding M8 of `analysis/july23-rule-concerns.md`. Until the department approves, a program may not replace an instructor who has resigned, change a practicum site, or move a classroom, and the rule as published binds the department to no period. The 60 calendar days drafted here are the period the rule already sets for curriculum approval in Subsection B of this section, and no source in this record proposes a period for this subsection. Three choices. Adopt the 60 days by analogy as drafted, set a shorter period for changes that do not touch curriculum, or provide that a reported change may be implemented if the department does not act within the period. Department of Health staff decide, because the period binds the department.

7.35.3.16 Requirements for third-party evaluators of educational programs

PART II

CURRENT LANGUAGE

Proposed rule as published July 23, 2026

PROPOSED AMENDMENT

Draft for review. Not filed, not adopted

Section heading and Subsection A, general requirements

7.35.3.16 REQUIREMENTS FOR THIRD-PARTY EVALUATORS OF EDUCATIONAL PROGRAMS: A. General requirements: A psilocybin educational program shall engage a qualified third-party evaluation team to conduct the evaluations required under this rule. The evaluation team shall consist of no fewer than three individuals who collectively meet all competency requirements set forth in this section.

Section heading and Subsection A, general requirements

7.35.3.16 REQUIREMENTS FOR THIRD-PARTY EVALUATORS OF EDUCATIONAL PROGRAMS: A. General requirements: A psilocybin educational program shall engage a qualified third-party evaluation team to conduct the evaluations required under this rule. The evaluation team shall consist of no fewer than three individuals who collectively meet all competency requirements set forth in this section. A program may compensate the evaluation team for conducting an evaluation required under this rule, and that compensation is not a conflict of interest under Subsection C of this section.

SOURCE Finding B3 of `analysis/july23-rule-concerns.md`, verified against the published rule. 7.35.3.16 NMAC Subsection A, page 10, requires that a program “shall engage a qualified third-party evaluation team”. Item (b) of Paragraph (2) of Subsection C of the same section, page 10, disqualifies an evaluator for “receiving compensation from... the program or organization being evaluated”, with no exception for the engagement itself. The amendment states that the engagement fee is not a disqualifying conflict. Both provisions are carried over from the July 9, 2026 draft. On why the evaluation exists, the July 17, 2026 Training and Education Committee transcript records that programs “also will have a need to have that third-party evaluation that’s looking to ensure that the curriculum is implemented with fidelity”.

Please review. The amendment cures the conflict by naming the engagement fee as something other than a disqualifying conflict. One alternative is to place the same words in item (b) of Paragraph (2) of Subsection C instead, and this document drafts that too so that the committee can adopt either or both. A second alternative is to have the department, rather than the program, engage and pay the team, which would remove the conflict entirely but requires a funding source that no document in this record identifies. The Training and Education Committee decides which route to send forward, and Department of Health staff decide whether the second is available.

Paragraph (2) of Subsection B, minimum qualifications

B. Minimum qualifications: Each evaluator shall possess at least three years of professional experience in one or more of the following fields, and the evaluation team as a whole shall demonstrate expertise covering all listed domains: (1) Psilocybin therapy practice, supported by a master's or doctoral degree in counseling, therapy, social work, public health, behavioral health, or a closely related field; (2) Medical and research practice, supported by a master's or doctoral degree in medicine, pharmacy, nursing, or another related clinical or scientific field; and (3) Educational curriculum development and evaluation, supported by a master's or doctoral degree in education, educational psychology, or a closely related field.

Paragraph (2) of Subsection B, minimum qualifications

B. Minimum qualifications: Each evaluator shall possess at least three years of professional experience in one or more of the following fields, and the evaluation team as a whole shall demonstrate expertise covering all listed domains: (1) Psilocybin therapy practice, supported by a master's or doctoral degree in counseling, therapy, social work, public health, behavioral health, or a closely related field; (2) Medical ~~and~~ or research practice, supported by a master's or doctoral degree in medicine, pharmacy, nursing, or another related clinical or scientific field; and (3) Educational curriculum development and evaluation, supported by a master's or doctoral degree in education, educational psychology, or a closely related field.

SOURCE Restoration. The July 9, 2026 draft read "Medical or research practice". The published rule at 7.35.3.16 NMAC Paragraph (2) of Subsection B, page 10, reads "Medical and research practice". On the published wording an evaluator in this domain needs both medical and research practice; on the July 9 wording either will do. The amendment restores the July 9 wording.

Please review. Finding M10 of `analysis/july23-rule-concerns.md`. Two questions sit on this paragraph. First, whether the change from "or" to "and" between the July 9 and July 23 drafts was deliberate. Two choices follow from the answer: keep the amendment, which restores the July 9 wording, or withdraw it if the narrowing was intended. Department of Health staff decide, because only the department knows whether the change was made on purpose. Second, whether a three-person team can in fact cover all three domains at three years of experience each in New Mexico. Two choices there as well: leave the standard as published, or lower it. No source in this record proposes a lower standard, so none is drafted. The Training and Education Committee decides whether to ask for one, because the committee is the body that will hear from programs that cannot find a qualifying team.

Subsection C, conflict of interest prohibitions

C. Conflict of -interest prohibitions: (1) All evaluators shall be free from actual or perceived conflicts of interest regarding the organization or curriculum under review. (2) The following conflicts, at minimum, shall disqualify an individual from serving as an evaluator: (a) Participation in the design, development, or teaching of the curriculum being evaluated; or (b) Holding a financial interest in, receiving compensation from, or serving in any governance or advisory capacity with the program or organization being evaluated.

Subsection C, conflict of interest prohibitions

C. Conflict ~~of -interest~~ of-interest prohibitions: (1) All evaluators shall be free from actual or perceived conflicts of interest regarding the organization or curriculum under review. (2) The following conflicts, at minimum, shall disqualify an individual from serving as an evaluator: (a) Participation in the design, development, or teaching of the curriculum being evaluated; or (b) Holding a financial interest in, receiving compensation from, or serving in any governance or advisory capacity with the program or organization being evaluated, other than compensation for conducting an evaluation required under this rule as provided in Subsection A of this section.

SOURCE Two changes. Correction: the heading at 7.35.3.16 NMAC Subsection C, page 10, reads "Conflict of -interest prohibitions"; the July 9, 2026 draft read "Conflict-of-interest prohibitions". Substance: the carve-out for the engagement fee, consequential on the amendment to Subsection A of the same section. Finding B3.

7.35.3.17 Educational programs; mentoring requirements; record-keeping

PART II

CURRENT LANGUAGE

Proposed rule as published July 23, 2026

PROPOSED AMENDMENT

Draft for review. Not filed, not adopted

Subsection A, mentoring sessions

A. All educational programs shall provide each practitioner and facilitator student with a minimum of 10 hours of mentoring sessions after graduation and after practicum hours are completed. The purpose of the mentoring sessions is to allow new graduates to consult with the educational program faculty regarding cases, patients, situations, and how they handle them. These mentoring sessions may not have additional associated charges beyond the cost of the educational program itself. The 10 hours are valid for up to 12 months after practicum hours are completed. If students request more than 10 hours of mentoring sessions, this will be at the purview of the educational program and may have an additional associated cost.

Subsection A, mentoring sessions

Not amended. This provision is reproduced because it carries a finding recorded below.

SOURCE Not amended. 7.35.3.17 NMAC Subsection A, pages 10 and 11, carried over from the July 9, 2026 draft. The Wilson working redline of July 25, 2026 rewrites this subsection: it retitles the section heading to read "CASE-CONSULTATION" in place of "MENTORING", replaces the 10 hours with "2 student case" consultations, adds that "Case presentations under consultation shall include discussion of individual case concerns, risk factors, supportive factors, treatment considerations, and recommendations for aftercare.", and strikes the last two sentences of the subsection. Her comment on the strike reads "want to keep this in or strike it? Would change hours to cases." and her comment on the added sentence reads "This probably does not need to live in regulation but can be part of a program guide for educational programs DOH could host and update regularly". This document draws no figure and no text from the redline here, because the placement of the consultation requirement is an open question in the practicum amendment document.

Please review. This subsection is left exactly as published, deliberately. The Wilson working redline would change the obligation from 10 hours of mentoring to 2 student case consultations, and the practicum amendment document holds open, for Dr. Metz, Ms. Wilson and Dr. Leeman, whether the consultation requirement is stated in hours, in cases, or in both, and whether it belongs at the proposed Subsection H of 7.35.3.19 NMAC, which exists only in that draft's re-lettering of that section, or at Subsection A of this section. As published, 7.35.3.19 NMAC runs A through G and has no Subsection H. Drafting either figure here would answer that question, so neither is drafted. Two choices, and they are theirs: state the requirement here, in which case this subsection is redrafted and the proposed 7.35.3.19 H carries a cross-reference, or state it at the proposed 7.35.3.19 H, in which case this subsection is struck. Dr. Metz, Ms. Wilson and Dr. Leeman decide which, and this section should be redrafted once they do. Findings M11 and M12 of `analysis/july23-rule-concerns.md` record the separate points that the obligation is unfunded and survives the program.

Subsection B, module evaluation without attendance

B. Educational programs shall offer an option for a student to complete the evaluation for each educational module (not including the New Mexico module) without attending the lessons or classes for the module. The cost to complete the evaluation for each module shall not be greater than one-fourth of the normal price of each of the educational modules. If the student receives a passing grade for a module evaluation, they will have been considered to have completed the module; and if a student does not receive a passing grade for a module evaluation, they will not have completed the module. The educational program may at its discretion allow students to re-take the evaluation without attending the lessons or classes for the modules. Students who utilize this option shall complete all other requirements for the educational program, including the New Mexico Module, certifications, simulated patient, and practicum requirements to graduate from the educational program.

Subsection B, module evaluation without attendance

Not amended. This provision is reproduced because it carries a finding recorded below.

SOURCE Not amended. 7.35.3.17 NMAC Subsection B, page 11, carried over from the July 9, 2026 draft. The Wilson working redline of July 25, 2026 strikes the whole subsection, and her comment on it reads “Strike, right? I don’t think we want this test out provision.”

Please review. Finding M12 of `analysis/july23-rule-concerns.md`: the price cap is stated as a fraction of the “normal price of each of the educational modules”, which assumes that programs price modules separately rather than pricing the program as a whole. Two choices. Strike the subsection, as the Wilson working redline proposes, or keep it and restate the cap so that it works for a program with a single price. No source in this record supplies a restated cap, so none is drafted. The Training and Education Committee decides whether the option survives at all, and only then is a restated cap worth drafting.

Part III. Locations and oversight

7.35.3.11, .20 and .21. Healing center and other approved location applications, the registration on which healing center staff authority depends, and department assessments.

7.35.3.11 Application process for healing centers and other approved locations

PART III

CURRENT LANGUAGE

Proposed rule as published July 23, 2026

PROPOSED AMENDMENT

Draft for review. Not filed, not adopted

Section heading and lead-in of Subsection A

7.35.3.11 APPLICATION PROCESS FOR HEALING CENTERS AND OTHER APPROVED LOCATIONS: (A) Applications for healing centers: An applicant seeking certification as a healing center shall submit a complete application through the electronic system designated by the department. A complete application shall include, at minimum, the following:

SOURCE Correction only. 7.35.3.11 NMAC Subsection A, page 5, is lettered "(A)"; Subsections B, C and D of the same section use the form "B.", "C." and "D." The only other section of the rule that uses parenthetical subsection letters is 7.35.3.14 NMAC, page 9, which is addressed in the practicum amendment document. No substance is changed.

Paragraph (11) of Subsection A, owners and employees

(11) A list of all owners or members of the board of directors, including corresponding contact information;

SOURCE Finding B5 of `analysis/july23-rule-concerns.md`, verified against the published rule. Subsection C of 7.35.3.14 NMAC, page 9, conditions the authority of healing center owners and employees to purchase, possess, sell or administer medical psilocybin on their being "registered with the department". The phrase "registered with the department" appears once in 7.35.3 NMAC, at that provision, and no registration for owners or employees exists in 7.35.3 NMAC or in 7.35.2 NMAC. The amendment supplies the registration at the point where the healing center already files the names: Paragraph (11) of Subsection A already requires "A list of all owners or members of the board of directors, including corresponding contact information", Paragraph (2) of the same subsection already requires "Names and contact information of primary contact person(s) and any affiliated practitioners or facilitators", and Subsection C of 7.35.3.14 NMAC already requires that the individual be "designated to engage in each activity by the healing center". Subsection C of 7.35.3.14 NMAC is new in the published rule; the July 9, 2026 draft contained no authorized-possession section. No figure is added.

Section heading and lead-in of Subsection A

7.35.3.11 APPLICATION PROCESS FOR HEALING CENTERS AND OTHER APPROVED LOCATIONS: ~~(A)~~ A. Applications for healing centers: An applicant seeking certification as a healing center shall submit a complete application through the electronic system designated by the department. A complete application shall include, at minimum, the following:

Paragraph (11) of Subsection A, owners and employees

(11) A list of all owners or members of the board of directors, including corresponding contact information; and a list of all owners and employees whom the healing center designates to purchase, possess, sell, or otherwise administer medical psilocybin products under Subsection C of 7.35.3.14 NMAC, including corresponding contact information. An owner or employee named on the list submitted under this paragraph is registered with the department for purposes of Subsection C of 7.35.3.14 NMAC;

Please review. The registration drafted here operates on the list the healing center files with its application, which fixes the position as at the date of the application. Nothing in 7.35.3.11 NMAC requires a healing center to update that list, and 7.35.3.11 NMAC has four subsections, A through D, none of which is a reporting duty; the only comparable duty in the rule is Subsection A of 7.35.3.15 NMAC, page 9, which applies to educational programs and not to healing centers. So an owner or employee who joins after certification would not be registered until the renewal under Subsection C of this section, which falls at two years. Three choices. Add a reporting duty for changes to the list, which is a new obligation that no source in this record proposes and which is therefore not drafted here. Rely on the two-year renewal. Or strike the registration condition from Subsection C of 7.35.3.14 NMAC instead of supplying a registration, which would leave the designation by the healing center as the only requirement. Department of Health staff decide, and the third choice has to be coordinated with the practicum amendment document because Subsection C of 7.35.3.14 NMAC is in that draft's scope. Whichever is chosen, the condition in Subsection C of 7.35.3.14 NMAC cannot be satisfied as published.

Paragraph (9) of Subsection B, proof of ownership

(9) Proof of ownership by the qualified patient or attending practitioner or facilitator of any property on which medical psilocybin administration sessions will be conducted, or a signed, written statement from the owner of such property acknowledging that the owner understands that persons will be participating in the medical psilocybin program on the premises, and what those persons are authorized to do within the terms of their certification;

Paragraph (9) of Subsection B, proof of ownership

(9) Proof of ownership by the qualified patient or attending practitioner or facilitator of any property on which medical psilocybin administration sessions will be conducted, or a signed, written statement from the owner of such property acknowledging that the owner understands that persons will be participating in the medical psilocybin program on the premises, and what those persons are authorized to do within the terms of their certification. This paragraph does not apply where the location is the residence of the qualified patient;

SOURCE Wilson working redline of July 25, 2026, comment on this paragraph: "seems like this will require EOL patients who are getting at home treatment to get permission from their landlord. We don't require that for any other in home healthcare and it's not appropriate to require that for EOL patients. People should not have to get their landlords permission to have certain healthcare." End-of-life care is a qualifying condition under Section 3(l) of the Medical Psilocybin Act and under 7.35.2.7 NMAC. 7.35.3.11 NMAC Paragraph (9) of Subsection B, page 6. Carried over from the July 9, 2026 draft.

Please review. The amendment exempts the qualified patient's own residence from the ownership proof and the owner's written statement. Two things to weigh. Subsection B of this section makes an other approved location available in part "for purposes of improving patient access in rural and frontier counties", and a patient's residence is the leading example the same subsection gives. Against that, the owner's statement is the only place in the rule where a property owner is told what will occur on the premises, so exempting the residence removes that notice for the location where sessions are most likely to be unsupervised by a healing center. Two choices. Adopt the exemption as drafted, or keep the requirement and allow the patient's own attestation in place of the owner's statement. The board decides, because the trade is patient access against notice to a third party.

Paragraph (10) of Subsection B, outdoor or natural environments

(10) For outdoor or natural environments: (a) A detailed description of the administration area, including identification of safe entrances and exits, and verification that the area is free from hazards; (b) An emergency safety and response plan; (c) Proof that emergency medical services can reliably be contacted from, and respond to, the location at which medical psilocybin administration sessions will occur; and (d) If the natural environment setting is 15 minutes or more away from emergency medical service response or a hospital, urgent care, or other emergency medical services, then there must be at least two individuals present who are not receiving treatment and who have: (i) Wilderness first aid certification; (ii) Wilderness first responder certification; or (iii) Licensure as a New Mexico emergency medical technician (e.g., EMT-basic, EMT-intermediate, EMT-paramedic);

Paragraph (10) of Subsection B, outdoor or natural environments

(10) For outdoor or natural environments: (a) A detailed description of the administration area, including identification of safe entrances and exits, and verification that the area is free from hazards; (b) An emergency safety and response plan; (c) Proof that emergency medical services can reliably be contacted from, and respond to, the location at which medical psilocybin administration sessions will occur; and (d) If the natural environment setting is 15 minutes or more away from emergency medical service response or a hospital, urgent care, or other emergency medical services, then there must be at least two individuals present who are not receiving treatment and who have: (i) Wilderness first aid certification; (ii) Wilderness first responder certification; or (iii) Licensure as a New Mexico emergency medical technician (e.g., EMT-basic, EMT-intermediate, EMT-paramedic); and (e) If the natural environment setting is 15 minutes or more away from emergency medical service response or a hospital, urgent care, or other emergency medical services, then there must be a basic first aid kit and an AED on site;

SOURCE Conforming amendment. Finding M16 of `analysis/july23-rule-concerns.md`, verified against the published rule. The outdoor requirements for a healing center at 7.35.3.11 NMAC Paragraph (22) of Subsection A, pages 5 and 6, run to item (e) and require "a basic first aid kit and an AED on site". The outdoor requirements for an other approved location at Paragraph (10) of Subsection B of the same section, pages 6 and 7, stop at item (d). The added item is the text of item (e) of Paragraph (22) of Subsection A, reproduced. No figure is added; the 15 minutes is the figure already in both paragraphs.

Please review. Two further questions on the same paragraph, neither answered here. The 15 minutes: the Wilson working redline comments on both places the figure appears, “Is this a reasonable limit? Everything feels at least 15 minutes away - should we say 30?” and “again - I think this should maybe be 30. Everything is 15 minutes away from everything.” Those are questions rather than a recommendation, and no source in this record supplies a figure, so the 15 minutes is left as published in both paragraphs. The two-person requirement: the redline adds at the healing center paragraph that “For the purposes of this section, an individual is a DOH-permitted practitioner, a DOH-premitted facilitator, or an individual who is a DOH registered student with an approved training program who has completed a minimum of 30 practicum hours and who is accompanied by a DOH-permitted practitioner.”, and the comment on it reads “Not sure where this goes - the 2 person universal requirement did not make it into the published rules, though DOH did confirm they mean for it to be universal.” Because the author records that she does not know where the provision belongs, it is not drafted. Ms. Wilson and Department of Health staff decide where it goes and whether the 15 minutes becomes 30.

Subsection D, certification period; approval and denial

D. Certification period; approval and denial: Certification of a healing center shall be effective on the date of department issuance and shall be valid for two years. Certification of other approved locations shall be effective on the date of department issuance and shall be valid for 90 days. If the department denies an application for a healing center or other approved location, the department shall provide notice of the denial within 30 calendar days in accordance with this rule. If the application is denied, the applicant may re-apply within six months. An applicant who is denied a second time may not re-apply for six months from the denial. An applicant whose application is denied may appeal the denial in accordance with this rule.

Subsection D, certification period; approval and denial

D. Certification period; approval and denial: Certification of a healing center shall be effective on the date of department issuance and shall be valid for two years. Certification of other approved locations shall be effective on the date of department issuance and shall be valid for 90 days, and may be renewed for additional 90-day terms on application to the department ^{90 from 7.35.3.11 D}. If the department denies an application for a healing center or other approved location, the department shall provide notice of the denial within 30 calendar days in accordance with this rule. If the application is denied, the applicant may re-apply ~~within no~~ earlier than six months after the date of the denial. An applicant who is denied a second time may not re-apply for six months from the denial. An applicant whose application is denied may appeal the denial in accordance with this rule.

SOURCE Two changes. Renewal: finding M22 of `analysis/july23-rule-concerns.md`, verified against the published rule. Subsection D gives an other approved location a 90-day certification and no renewal route, while Subsection C of the same section supplies a renewal route for healing centers only. The 90 days in the amendment is the figure already in Subsection D. Re-application: the same conforming change as 7.35.3.10 NMAC Subsection C. 7.35.3.11 NMAC Subsection D, page 7. Carried over from the July 9, 2026 draft.

Please review. The renewal drafted here repeats the 90 days the subsection already sets, so nothing new is introduced, but three questions remain. Whether 90 days is the right term at all, which no source in this record addresses. Whether a renewal application needs its own contents, or whether the application under Subsection B of this section is resubmitted. Whether the department is bound to any period in deciding a renewal, since Subsection D binds it to 30 calendar days only for a denial. Department of Health staff decide all three, because each one binds the department, and the Training and Education Committee should say whether a 90-day term with renewal is workable for a practicum site.

7.35.3.20 Requirements for healing centers and other approved locations

PART III

CURRENT LANGUAGE

Proposed rule as published July 23, 2026

PROPOSED AMENDMENT

Draft for review. Not filed, not adopted

Section heading, lead-in, and Subsection A**7.34.3.20 REQUIREMENTS FOR HEALING CENTERS AND OTHER APPROVED**

LOCATIONS: A healing center and a registrant of another approved location shall comply with the following requirements: A. A healing center and a registrant of another approved location shall maintain a list of all qualified patients who are intended to utilize, and who have utilized, the location for administration sessions.

Section heading, lead-in, and Subsection A

Not amended. This provision is reproduced because it carries a finding recorded below.

SOURCE Not amended here. The section heading reads 7.34.3.20 and its correction is drafted in the practicum amendment document, so it is not duplicated. Recorded for two other things. Terminology: the lead-in and Subsection A, page 13, use “a registrant of another approved location”, and the term “registrant” appears 10 times in 7.35.3.20 NMAC and nowhere else in the rule, while Subsection B of 7.35.3.11 NMAC, page 6, issues a certification for such a location rather than a registration; the definition drafted at 7.35.3.7 NMAC ties the two together. Patient roster: finding M18. Carried over from the July 9, 2026 draft.

Please review. Two questions, neither drafted here. Terminology: this section calls the holder of an other approved location certification a registrant 10 times, and Subsection B of 7.35.3.11 NMAC issues a certification. Two choices. Adopt the definition drafted at 7.35.3.7 NMAC, which ties the two words together and leaves this section untouched, or conform all 10 occurrences here to certificant. Department of Health staff decide which, and the second choice requires 10 edits this document does not make. Patient roster: finding M18 of `analysis/july23-rule-concerns.md` records that Subsection A requires a list of all qualified patients intended to use and who have used the location, which read with Subsection A of 7.35.3.21 NMAC allows the department to obtain patient identities and interview patients. Whether the roster should instead be kept in a form that does not identify patients to the department is a question for the board, and it turns on the department's data needs, which Department of Health staff would have to state.

7.35.3.21 Department evaluation and assessment

PART III

CURRENT LANGUAGE

Proposed rule as published July 23, 2026

PROPOSED AMENDMENT

Draft for review. Not filed, not adopted

Subsection A, authority to conduct assessments

A. Authority to conduct assessments: The department or its designee may conduct remote or on-site assessments of a psilocybin education program, healing center or other approved location, with or without prior notice, during normal business hours, for the purpose of determining compliance with the Medical Psilocybin Act and this rule. In conducting assessments, the department may review documentation, inspect premises and equipment, and interview staff and patients. Exception: except in emergency circumstances, the department or its designee shall not conduct an on-site assessment during a patient session or at a patient's home without prior written consent from the patient(s).

Subsection A, authority to conduct assessments

Not amended. This provision is reproduced because it carries a finding recorded below.

SOURCE Not amended. Recorded for two things. Patient interviews: Subsection A, page 15, permits the department to "interview staff and patients", subject to the exception in the same subsection for an on-site assessment during a patient session or at a patient's home. Frequency: 7.35.3.21 NMAC sets no interval at which assessments occur, while 7.35.3.16 NMAC Subsection E, page 10, requires a third-party evaluation report without stating how often. Carried over from the July 9, 2026 draft, where the section was titled "PSILOCYBIN EDUCATIONAL PROGRAMS, HEALING CENTERS, OTHER APPROVED LOCATIONS; EVALUATION AND ASSESSMENT BY THE DEPARTMENT".

Please review. Two questions, neither drafted here. Patient interviews: the exception in this subsection protects a patient during a session and at home, and does not otherwise limit whom the department may interview or require that the patient consent. Whether an additional consent requirement should apply to any patient interview is a question for the board. Frequency: the rule does not say how often the department assesses a program, a healing center or an other approved location, and it does not say how often a third-party evaluation must be repeated once the initial one is filed under Paragraph (20) of Subsection A of 7.35.3.12 NMAC. Finding M10 of `analysis/july23-rule-concerns.md` records the second point. No source in this record supplies an interval, so none is drafted. Department of Health staff decide both, because each is a demand on department capacity.

Part IV. Patients, applicants and process

7.35.3.8, .9, .13 and .22 through .28. Enrollment and certification applications, complaints, review of denials, and discipline.

7.35.3.8 Patient enrollment application process

PART IV

CURRENT LANGUAGE

Proposed rule as published July 23, 2026

PROPOSED AMENDMENT

Draft for review. Not filed, not adopted

Subsection D, review of a denied patient application

D. A person whose application for patient enrollment is denied for failure to complete an application or failure to meet a submittal requirement of this rule may request a record review to be conducted by the department, in accordance with this rule.

Subsection D, review of a denied patient application

D. A person whose application for patient enrollment is denied for failure to complete an application or failure to meet a submittal requirement of this rule may request ~~a record review to be conducted by the department~~ an informal administrative review under 7.35.3.25 NMAC, in accordance with this rule.

SOURCE Conforming amendment. 7.35.3.8 NMAC Subsection D, page 2, gives a denied patient applicant "a record review to be conducted by the department". The rule provides no proceeding called a record review. 7.35.3.25 NMAC, page 16, provides an informal administrative review for "An applicant for patient enrollment whose application has been denied", which is the same class of person. The amendment names that proceeding and cites it. The July 9, 2026 draft titled the corresponding section "DENIAL OF AN INITIAL PATIENT APPLICATION; RECORD REVIEW", and its Subsection A read that applicants "may request a record review from the department", so the two names described one proceeding in that draft. Section 7.35.3.8 is new in the published rule.

Please review. Finding M23 of `analysis/july23-rule-concerns.md`. Three provisions describe review of a denial, and their scopes do not line up. This subsection covers a patient applicant denied for an incomplete application or a missed submittal requirement. Subsection A of 7.35.3.25 NMAC covers any patient applicant whose application has been denied. Paragraph (7) of Subsection C of 7.35.3.27 NMAC gives a hearing to an applicant denied for any reason other than an incomplete application or a missed submittal requirement. Read together, the informal review reaches every denial and the hearing reaches every denial except the two that this subsection covers, so a patient applicant denied on the merits may have both. The amendment drafted here only fixes the name of the proceeding and leaves the overlap alone. Two choices on the overlap. Narrow Subsection A of 7.35.3.25 NMAC to the grounds this subsection names, or state that the informal review and the hearing are alternatives and which one a patient applicant elects. Department of Health staff and department counsel decide, because the answer affects what process a denied applicant is owed.

7.35.3.9 Certifying clinician, practitioner, and facilitator application process

PART IV

CURRENT LANGUAGE

Proposed rule as published July 23, 2026

PROPOSED AMENDMENT

Draft for review. Not filed, not adopted

Paragraphs (5) and (6) of Subsection A, re-application after denial

(5) If the application is denied, the applicant may re-apply within six months. (6) An applicant who is denied a second time may not re-apply for six months from the denial.

Paragraphs (5) and (6) of Subsection A, re-application after denial

(5) If the application is denied, the applicant may re-apply ~~within~~ **no earlier than** six months **after the date of the denial**. (6) An applicant who is denied a second time may not re-apply for six months from the denial.

SOURCE Conforming amendment. Same defect and same fix as 7.35.3.10 NMAC Subsection C. 7.35.3.9 NMAC Paragraphs (5) and (6) of Subsection A, page 2.

Subsection D, certifying clinician application requirements

D. Certifying clinician application requirements: An applicant for certification as a certifying clinician shall additionally submit the following, using the electronic system designated by the department: (1) Documentation of current professional license to practice in New Mexico (e.g. MD, NP); (2) NM controlled substance number; (4) Documentation of HIPAA certification completed within the preceding two years; (5) Completed W-9 form; and (6) Such additional information as the department may reasonably require.

Subsection D, certifying clinician application requirements

D. Certifying clinician application requirements: An applicant for certification as a certifying clinician shall additionally submit the following, using the electronic system designated by the department: (1) Documentation of current professional license to practice in New Mexico (e.g. MD, NP); (2) NM controlled substance number; ~~(4)~~ **(3)** Documentation of HIPAA certification completed within the preceding two years; ~~(5)~~ **(4)** Completed W-9 form; and ~~(6)~~ **(5)** Such additional information as the department may reasonably require.

SOURCE Correction. Finding N3 of `analysis/july23-rule-concerns.md`, verified against the published rule. The paragraphs of 7.35.3.9 NMAC Subsection D, page 3, run (1), (2), (4), (5), (6). There is no (3). The amendment renumbers so that the list is consecutive and adds no item. The paragraph reproduced here contains the controlled substance number requirement at Paragraph (2), which is out of scope for this document by decision; that paragraph is reproduced as published and is not amended.

Please review. The renumbering assumes that the gap is a numbering error rather than the trace of a deleted requirement. Section 7.35.3.9 has no counterpart in the July 9, 2026 draft, so this record cannot show what, if anything, stood at Paragraph (3). Two choices. Renumber as drafted, or restore the missing requirement if one was intended. Department of Health staff decide, because only the department knows what the paragraph held.

7.35.3.13 Requirements and prohibitions for certifying clinicians, practitioners, and facilitators

PART IV

CURRENT LANGUAGE

Proposed rule as published July 23, 2026

PROPOSED AMENDMENT

Draft for review. Not filed, not adopted

Section heading

7.34.3.13 REQUIREMENTS AND PROHIBITIONS FOR CERTIFYING CLINICIANS, PRACTITIONERS, AND FACILITATORS:

Section heading

~~7.34.3.13~~ **7.35.3.13** REQUIREMENTS AND PROHIBITIONS FOR CERTIFYING CLINICIANS, PRACTITIONERS, AND FACILITATORS:

SOURCE Correction only. Finding N1 of `analysis/july23-rule-concerns.md`, verified against the published rule. The heading at page 8 reads 7.34.3.13; the history note at the end of the same section reads “[7.35.3.13 NMAC, xx/xx/2026]”. Chapter 34 of Title 7 is a different chapter. Five headings in the rule read 7.34.3: those of 7.35.3.13, 7.35.3.14, 7.35.3.20, 7.35.3.23 and 7.35.3.25. The headings of 7.35.3.14 and 7.35.3.20 are corrected in the practicum amendment document; the other three are corrected here.

Subsection B, facilitators; scope of work

B. Facilitators; scope of work: A facilitator is authorized to work alongside a practitioner during medical psilocybin services. A facilitator works under the direct supervision of a practitioner, and provides peer support to qualified patients, as well as logistical and administrative support to the practitioner and to the healing center. A facilitator shall not perform any patient care outside this scope, unless another license held by the facilitator permits it.

Subsection B, facilitators; scope of work

Not amended. This provision is reproduced because it carries a finding recorded below.

SOURCE Not amended. Recorded because it is the source of the facilitator definition drafted at 7.35.3.7 NMAC and because of finding M15 of `analysis/july23-rule-concerns.md`. 7.35.3.13 NMAC Subsection B, page 8. Carried over from the July 9, 2026 draft.

Please review. Finding M15 of `analysis/july23-rule-concerns.md`. This subsection puts a facilitator under the direct supervision of a practitioner and limits the facilitator to peer support and to logistical and administrative support. Three provisions sit against it, and all three are in the practicum amendment document rather than this one. Subsection B of 7.35.3.14 NMAC, page 9, lets a facilitator possess psilocybin products and provide them to qualified patients. Paragraph (5) of Subsection H of 7.35.3.20 NMAC, page 14, permits a group session staffed by one facilitator or qualified student for every two patients with one practitioner for every eight patients, so a facilitator may be the only certificant with a given patient. Subsection C of 7.35.3.19 NMAC, page 13, has practitioners supervising facilitators during administration sessions as part of the practicum. Whether “direct supervision” means presence in the room, presence at the location, or availability is not stated anywhere in the rule. Three choices. Define direct supervision in this part, which the definitions drafted at 7.35.3.7 NMAC could carry; narrow the facilitator authority in the three provisions named above to match this subsection; or widen this subsection to match them. No source in this record chooses among the three, so none is drafted. The Training and Education Committee and Department of Health staff decide the meaning, and because the conflicting provisions belong to the practicum draft, any amendment has to be coordinated with that document rather than made here.

7.35.3.23 Prohibition against dual ownership in certificant and permittee

PART IV

CURRENT LANGUAGE	PROPOSED AMENDMENT
Proposed rule as published July 23, 2026	Draft for review. Not filed, not adopted
Entire section, dual ownership 7.34.3.23 PROHIBITION AGAINST DUAL OWNERSHIP IN CERTIFICANT AND PERMITTEE: A certificant or a person who holds an ownership interest in a certificant shall not hold an ownership interest in a permittee. SOURCE Correction only. Finding N1. The heading at page 15 reads 7.34.3.23; the history note at the end of the same section reads "[7.35.3.23 NMAC, xx/xx/2026]". The substance is not amended. "Permittee" is defined at 7.35.2.7 NMAC as a psilocybin producer or psilocybin testing laboratory that holds a permit, so the prohibition operates on the producer and laboratory side without further definition. Carried over from the July 9, 2026 draft.	Entire section, dual ownership 7.34.3.23 7.35.3.23 PROHIBITION AGAINST DUAL OWNERSHIP IN CERTIFICANT AND PERMITTEE: A certificant or a person who holds an ownership interest in a certificant shall not hold an ownership interest in a permittee.

7.35.3.24 Complaints to the department

PART IV

CURRENT LANGUAGE

Proposed rule as published July 23, 2026

PROPOSED AMENDMENT

Draft for review. Not filed, not adopted

Entire section, complaints to the department

7.35.3.24 COMPLAINTS TO THE DEPARTMENT: A qualified patient or certificant may submit a complaint to the department regarding a certificant in the medical psilocybin program, or regarding any activity or concern related to the medical psilocybin program. The complaint should be in writing, addressed to the medical psilocybin program, and submitted by U.S. certified mail or through the electronic system designated by the department electronic system. The department may request additional information for purposes of its own inquiry. A complaint shall include the name and contact information for the complainant; and a statement of the nature of the complaint and any individuals who may be involved. The department may, within its discretion, choose to conduct a subsequent inquiry, and may request additional information from the complainant for this purpose.

Entire section, complaints to the department

7.35.3.24 COMPLAINTS TO THE DEPARTMENT: A qualified patient or certificant may submit a complaint to the department regarding a certificant in the medical psilocybin program, or regarding any activity or concern related to the medical psilocybin program. The complaint should be in writing, addressed to the medical psilocybin program, and submitted by U.S. ~~certified~~ mail or through the electronic system designated by the department ~~electronic system~~. The department may request additional information for purposes of its own inquiry. A complaint shall include the name and contact information for the complainant, which shall remain confidential; and a statement of the nature of the complaint and any individuals who may be involved. The department may, within its discretion, choose to conduct a subsequent inquiry, and may request additional information from the complainant for this purpose.

SOURCE Three changes, all restorations or corrections. Confidentiality: finding M17 of `analysis/july23-rule-concerns.md`, verified against the July 9, 2026 draft, which required that a complaint include the "Name and contact information for the complainant which will remain confidential". The published section, page 15, requires the name and contact information and drops the assurance. Mail: the July 9, 2026 draft read that a complaint "should be in writing and submitted by U.S. mail or through the electronic" system; the published section requires "U.S. certified mail". Duplication: the published section reads "through the electronic system designated by the department electronic system". Section 7.35.3.24 is new in the published rule; the July 9, 2026 draft carried the same substance as an unnumbered list headed "Complaint Reporting".

Please review. Two questions the amendment does not reach. Standing: the July 9, 2026 draft provided that "Patients or staff may lodge a complaint with the department." The published section allows a complaint from a qualified patient or a certificant, so an employee of a healing center who is neither has no route under this section. Two choices. Restore standing for staff, or leave the section as published and rely on the adverse health event reporting in Subsection L of 7.35.3.20 NMAC. Scope of the confidentiality restored here: the July 9 wording protected the complainant's name and contact information and said nothing about disclosure to the certificant complained of, or about the application of the Inspection of Public Records Act. Department of Health staff and department counsel decide both, and the board should be told whether a complainant can be assured of confidentiality before the rule is filed.

7.35.3.25 Informal administrative review of denied patient applications

PART IV

CURRENT LANGUAGE

Proposed rule as published July 23, 2026

PROPOSED AMENDMENT

Draft for review. Not filed, not adopted

Section heading and Subsection A

7.34.3.25 INFORMAL ADMINISTRATIVE REVIEW OF DENIED PATIENT APPLICATIONS:

A. Administrative review: An applicant for patient enrollment whose application has been denied may request an informal administrative review from the department.

Section heading and Subsection A~~7.34.3.25~~ **7.35.3.25** INFORMAL ADMINISTRATIVE REVIEW OF DENIED PATIENT

APPLICATIONS: A. Administrative review: An applicant for patient enrollment whose application has been denied may request an informal administrative review from the department.

SOURCE Correction only. Finding N1. The heading at page 16 reads 7.34.3.25; the history note at the end of the same section reads "[7.35.3.25 NMAC, xx/xx/2026]". The substance is not amended.

Paragraph (1) of Subsection D, content and timeline

(1) Content and timeline: The administrative review shall be completed, and a decision setting forth the reasons for the decision and the evidence upon which the decision is based, no later than 15 calendar days from the date that the program receives the written request for a record review.

Paragraph (1) of Subsection D, content and timeline(1) Content and timeline: The administrative review shall be completed, and a decision shall be issued setting forth the reasons for the decision and the evidence upon which the decision is based, no later than 15 calendar days from the date that the program receives the written request for ~~a record review~~ an informal administrative review.

SOURCE Two corrections. Grammar: finding N4 of `analysis/july23-rule-concerns.md`, verified against the published rule. The sentence at page 16 has no verb governing the decision, and is carried over verbatim from the July 9, 2026 draft. The amendment supplies the verb and changes nothing else. Terminology: the same sentence refers to "the written request for a record review", while Subsection A of the same section and Paragraph (1) of Subsection B call the proceeding an administrative review. The 15 calendar days are not amended.

Please review. Two questions on this section that the corrections do not reach. The administrative review committee: Subsection C of this section and Paragraph (2) of Subsection D assign the review to "the administrative review committee", and nothing in 7.35.3 NMAC or 7.35.2 NMAC constitutes that committee, states who sits on it, or says how it is appointed. The term appears four times in the rule, all in this section, and it is carried over from the July 9, 2026 draft. Judicial review: Subsection E provides that "Except as otherwise provided by law, there shall be no right to judicial review of a decision by the administrative review committee." and the Wilson working redline comments on that subsection "they really went in the opposite direction of due process here..." Neither is drafted, because constituting a committee and limiting judicial review are both beyond what a correction can carry, and no source in this record proposes terms for either. Department of Health staff must say who the committee is, and department counsel must say whether Subsection E is within the department's authority. The board should have both answers before the rule is filed.

7.35.3.27 Disciplinary actions and appeal process

PART IV

CURRENT LANGUAGE

Proposed rule as published July 23, 2026

PROPOSED AMENDMENT

Draft for review. Not filed, not adopted

Paragraph (1) of Subsection C, persons who may request a hearing

C. Persons who may request a hearing: The following persons may request a hearing to contest an action or proposed action of the department, in accordance with this rule: (1) a certified patient; (2) a certified practitioner; (3) a certifying clinician who was certified by the department; (4) a certified facilitator; (5) a certified healing center or other approved location; (6) a certified educational program; and (7) an applicant for enrollment or certification as any of the foregoing, whose application is denied for any reason other than failure to submit a completed application or failure to meet a submittal requirement of this rule.

SOURCE Conforming amendment. Paragraph (1) of Subsection C, page 17, reads "a certified patient". A patient is enrolled rather than certified: 7.35.3.8 NMAC Subsection E, page 2, provides for "A qualified patient's enrollment", 7.35.3.22 NMAC, page 15, distinguishes "a qualified patient or certificant", and 7.35.2.7 NMAC defines "qualified patient". The subsection is reproduced in full because it is the source of the certificant definition drafted at 7.35.3.7 NMAC. Carried over from the July 9, 2026 draft.

Paragraph (1) of Subsection C, persons who may request a hearing

C. Persons who may request a hearing: The following persons may request a hearing to contest an action or proposed action of the department, in accordance with this rule: (1) a ~~certified~~ qualified patient; (2) a certified practitioner; (3) a certifying clinician who was certified by the department; (4) a certified facilitator; (5) a certified healing center or other approved location; (6) a certified educational program; and (7) an applicant for enrollment or certification as any of the foregoing, whose application is denied for any reason other than failure to submit a completed application or failure to meet a submittal requirement of this rule.

Paragraph (8) of Subsection B, grounds for disciplinary action

(8) for certifying clinicians and practitioners: any determination by the individual's licensing body that the clinician has engaged in unprofessional or dishonorable conduct;

SOURCE Correction. Paragraph (8) of Subsection B, page 17, opens "for certifying clinicians and practitioners" and then refers only to "the clinician". The amendment names both classes the paragraph applies to. Carried over from the July 9, 2026 draft.

Paragraph (8) of Subsection B, grounds for disciplinary action

(8) for certifying clinicians and practitioners: any determination by the individual's licensing body that the ~~clinician~~ certifying clinician or practitioner has engaged in unprofessional or dishonorable conduct;

Subsection M, continuances

M. Continuances: The hearing examiner may grant a continuance for good cause shown. A motion to continue a hearing shall be made at least 10 calendar days before the hearing date.

SOURCE Correction. Finding N4. Subsection M, page 18, reads "The hearing examiner may grant a continuance for good cause shown". Every other subsection of 7.35.3.27 NMAC that names the presiding official calls that official the hearing officer, including Paragraph (1) of Subsection E, page 17: "All hearings held pursuant to this section shall be conducted by a hearing officer appointed by the secretary." The 10 calendar days are not amended. Carried over from the July 9, 2026 draft.

Subsection M, continuances

M. Continuances: The hearing ~~examiner~~ officer may grant a continuance for good cause shown. A motion to continue a hearing shall be made at least 10 calendar days before the hearing date.

Please review. Finding N5 of `analysis/july23-rule-concerns.md`, which the correction to this subsection does not reach. Subsection A of this section newly allows the department to suspend a certification immediately. After a suspension, Subsection F allows the hearing to be held up to 60 calendar days after the request, Paragraph (2) of Subsection O allows the hearing officer 30 calendar days after the last submission to recommend, and Paragraph (3) of Subsection O allows the secretary 45 calendar days after receipt to decide, so a suspended certificant can be out of practice for more than 135 days before a final decision. The rule provides no expedited track. Two choices. Add an expedited schedule for a hearing that follows an immediate suspension, or leave the timeline as published. No source in this record supplies a shorter period, so none is drafted. Department of Health staff and department counsel decide, because the periods bind the department and the secretary.

Addendum A Terms this part uses and no part defines

7.35.3.7 NMAC provides in full: "The definitions in 7.35.2.7 NMAC apply to this part." Every count in the table below is produced from the text layer of the published rule and of 7.35.2 NMAC each time this document is built, section by section, so no count in it is transcribed by hand. Column headings are section numbers within each part. The first block is terms used in 7.35.3 NMAC that 7.35.2.7 NMAC does not define. The second block, shaded, is the terms 7.35.2.7 NMAC does define, shown for contrast.

Term in 7.35.3 NMAC	1	2	3	4	5	6	7	8	9	10	11	12	13	14	15	16	17	18	19	20	21	22	23	24	25	26	27	28	Part 3	Part 2	
educational program	.	1	.	.	.	1	.	7	8	.	28	.	7	.	.	3	3	13	1	2	1	3	.	.	.	.	1	3	.	68	0
facilitator	.	1	.	.	.	1	.	.	5	5	3	1	14	5	.	.	2	15	4	3	.	.	.	.	.	1	4	.	65	0	
healing center	.	1	.	.	.	.	.	.	.	.	10	1	2	10	.	.	.	.	1	21	3	.	.	.	.	1	3	.	54	0	
certifying clinician	.	.	.	.	.	1	.	16	5	4	.	.	11	.	.	.	.	10	.	.	.	.	.	.	.	1	4	.	53	0	
other approved location	.	1	.	.	.	.	.	.	.	.	6	.	1	3	.	.	.	.	1	14	3	.	.	.	.	1	3	.	34	0	
practicum	.	.	3	.	.	.	.	.	3	4	.	1	.	.	1	.	5	.	13	2	.	.	.	.	.	.	.	.	29	0	
student	.	.	.	.	.	.	.	.	.	.	.	5	.	.	.	3	12	.	5	3	.	.	.	.	.	.	.	.	28	0	
certificant	.	.	.	.	.	.	.	.	.	.	.	.	.	.	.	.	.	.	.	2	3	3	2	.	.	1	.	11	0		
registrant	.	.	.	.	.	.	.	.	.	.	.	.	.	.	.	.	.	.	10	.	.	.	.	.	.	.	.	.	10	0	
didactic	.	.	.	.	.	.	.	.	.	4	.	.	.	.	.	.	.	4	2	.	.	.	.	.	.	.	.	.	10	0	
adverse health event	.	.	.	.	.	.	.	.	.	.	1	.	1	.	.	.	.	.	.	4	.	.	.	.	.	.	.	.	6	0	
administrative review committee	.	.	.	.	.	.	.	.	.	.	.	.	.	.	.	.	.	.	.	.	.	.	.	.	4	.	.	.	4	0	
graduate	.	.	.	.	.	.	.	.	.	.	.	1	.	.	.	.	2	.	1	.	.	.	.	.	.	.	.	.	4	0	
simulated patient	.	.	.	.	.	.	.	.	.	.	.	.	.	.	.	.	1	2	1	.	.	.	.	.	.	.	.	.	4	0	
medical psilocybin services	.	.	.	.	.	.	.	.	.	.	.	.	1	.	.	.	.	1	.	1	.	.	.	.	.	.	.	.	3	0	
equity and access fund	.	.	.	.	.	.	.	1	.	.	.	.	.	.	.	.	.	.	.	.	.	.	.	.	.	.	.	.	1	0	
Defined in 7.35.2.7 NMAC, for contrast	.	.	.	.	.	.	.	.	.	.	.	.	.	.	.	.	.	.	.	.	.	.	.	.	.	.	.	.	.	.	
practitioner	.	1	.	.	.	1	.	.	5	5	3	1	12	6	.	.	2	14	6	3	.	.	.	.	.	1	5	.	66	12	
clinician	.	1	.	.	.	.	5	.	.	.	.	.	4	.	.	.	.	.	.	.	.	.	.	.	1	.	2	.	13	4	
qualified patient	.	.	.	.	.	1	.	3	.	.	2	.	4	3	.	.	.	.	3	.	3	.	1	.	1	1	.	22	6		
administration session	.	.	.	.	.	.	.	.	.	.	5	.	.	3	.	.	.	1	.	7	.	1	.	.	.	.	.	17	4		
approved location	.	1	.	.	.	1	.	.	.	.	6	1	1	3	.	.	.	.	1	15	3	.	.	.	.	1	3	.	37	2	
guide	.	.	.	.	.	.	.	.	.	.	.	.	.	.	.	.	.	.	.	.	.	.	.	.	.	.	.	.	0	3	

What the table shows. Sixteen terms carrying regulatory consequence in this part are defined in neither part. Six of them are defined in the draft at 7.35.3.7 NMAC, because a single provision of the published rule supplies the content: certificant, facilitator, other approved location, practicum, registrant of another approved location, and student. Ten are not, and the two largest are healing center and certifying clinician. "Guide", the one defined term in 7.35.2.7 NMAC for an individual who assists practitioners during administration sessions, appears nowhere in 7.35.3 NMAC, while "facilitator", which 7.35.2.7 NMAC does not define, appears throughout it. 7.35.2 NMAC was adopted effective June 23, 2026, so amending 7.35.2.7 NMAC is a separate rulemaking and any term this part needs has to be carried in this part.

The Act. Section 3(B) of the Medical Psilocybin Act defines a clinician as an approved health care provider licensed in New Mexico who holds a permit from the department to provide medical services to qualified patients. 7.35.2.7 NMAC defines "Clinician" in the same terms except that it reads certification where the Act reads permit, and defines "Permit" as the authorization to operate as a psilocybin producer or psilocybin testing laboratory. Section 5 of the Act exempts a

producer, a clinician and a qualified patient from arrest, prosecution and penalty. Neither the Act nor 7.35.2.7 NMAC uses the word facilitator. The consequence is stated at 7.35.3.7 NMAC in the redline above.

Addendum A, continued

The material for a definition of each term, assembled from the sources

For each term the table counts, what follows gathers the provisions of 7.35.3 NMAC, 7.35.2 NMAC and the Medical Psilocybin Act that bear on the term, quoted exactly and cited, so that a definition can be decided from assembled material rather than composed from scratch. Two of the sixteen, other approved location and practicum, are defined in the redline at 7.35.3.7 NMAC because a single provision supplies the content; they are marked below, and the source of each is quoted. Nothing below proposes language for the remaining ten, and none of the sixteen terms appears anywhere in the Medical Psilocybin Act.

healing center. The most-used undefined term, and no provision states what a healing center is. What the rule supplies: it is certified on application, 7.35.3.11 NMAC Subsection A, page 5, "An applicant seeking certification as a healing center shall submit a complete application"; it is an organization with owners, a board of directors and employees, Paragraph (11) of the same subsection; and it is a place where administration sessions occur and psilocybin products are handled, 7.35.3.14 NMAC, page 9, under which "Owners and employees of healing centers who are registered with the department may purchase medical psilocybin products from permitted producers, may possess medical psilocybin products, and may sell or otherwise administer those products to qualified patients in administration sessions conducted at the healing center or other approved locations", and "A healing center may only sell or otherwise provide psilocybin products that are obtained from permitted producers." Its operating duties are stated at 7.35.3.20 NMAC, pages 13 to 15, including the patient list at Subsection A and the reporting duty at Subsection L. Its location is what an other approved location is defined against: other approved locations "are physical locations at which medical psilocybin is intended to be consumed, that are not locations of healing centers", 7.35.3.11 NMAC Subsection B, page 6. 7.35.2 NMAC does not use the term; its nearest defined term is "Approved location", meaning "a location approved by the department for psilocybin administration sessions". The Act does not use the term and speaks instead of settings: Section 5(B)(2) makes lawful the conduct of a clinician administering or a qualified patient taking psilocybin "in an approved setting", and Section 7(A)(3) directs the department to establish "treatment protocols, including patient selection criteria, medical service standards, dosage standards and approved settings for administration of psilocybin to patients". Whether a healing center is one of the Act's approved settings is stated nowhere in this record.

certifying clinician. No provision states what a certifying clinician is, and the rule uses the bare word clinician in its scope section while using the two-word term everywhere else. What the rule supplies: the certifying clinician stands at the door of the program, since a patient application "shall be submitted by a patient and the patient's certifying clinician", 7.35.3.8 NMAC lead-in, page 1; the work is certifying patients, and "In order to certify a patient, a certifying clinician must have an actual clinician-patient relationship with the patient.", 7.35.3.13 NMAC Subsection A, page 8; an applicant must hold a license, "Documentation of current professional license to practice in New Mexico (e.g. MD, NP)", 7.35.3.9 NMAC Subsection D, page 3; and "All certifying clinicians shall complete a certifying clinician module", 7.35.3.18 NMAC Subsection B, page 11. What the other sources supply: Section 3(B) of the Act defines "clinician" as "an approved health care provider licensed in New Mexico who holds a permit from the department to provide medical services to qualified patients", and 7.35.2.7 NMAC defines "Clinician" in the same words except that it reads certification where the Act reads permit. Whether the certifying clinician of this part is the clinician that Section 3(B) of the Act and 7.35.2.7 NMAC define is stated nowhere in this record.

educational program. Regulated as a certificant, never described. What the rule supplies: it is certified on application, "An applicant seeking certification as a psilocybin educational program shall submit a complete application through the electronic system designated by the department.", 7.35.3.12 NMAC Subsection A, page 7; its application must include, at Paragraph (18) of that subsection, "A plan demonstrating how the program will ensure that enrolled students complete all required components and graduate within two years of enrollment"; and it is one of the classes that may relinquish a certification, 7.35.3.26 NMAC, page 16. 7.35.2 NMAC does not use the term. The Act does not use the term; its only training provision is Section 7(A)(2), which directs the department to establish "necessary initial and ongoing training for producers and clinicians", naming neither practitioners, facilitators, students nor programs.

other approved location. Defined in the redline at 7.35.3.7 NMAC. One provision supplies the content, which is why this term is drafted: 7.35.3.11 NMAC Subsection B, page 6, provides that "Other approved locations are physical locations at which medical psilocybin is intended to be consumed, that are not locations of healing centers." The same subsection supplies the "temporary department certification", the purposes, "if a qualified patient is unable to be physically transported to a healing center, for other reasons of medical necessity, for purposes of improving patient access in rural and frontier counties, or to enable treatment to occur in a natural environment setting", and the examples, "Such locations may include a patient's residence, or another temporary location." 7.35.2.7 NMAC defines "Approved location" but not this longer term, and the Act does not use it.

practicum. Defined in the redline at 7.35.3.7 NMAC. One provision supplies the content, which is why this term is drafted: 7.35.3.19 NMAC Subsection A, page 12, provides that an individual who seeks to become certified as a practitioner or facilitator "shall participate in supervised practice training, otherwise referred to as a" practicum. That section belongs to the practicum amendment draft, whose working draft v9 amends the hour figures in the same subsection and keeps the quoted phrase; Addendum B records the dependency. In this document's scope the word appears in the reporting duty for "Any modification,

termination, or addition of practicum site agreements", 7.35.3.15 NMAC Subsection A, page 9, in the mentoring obligation that runs "after graduation and after practicum hours are completed", 7.35.3.17 NMAC Subsection A, page 10, and in the waiver reproduced at Paragraph (1) of Subsection D of 7.35.3.10 NMAC, page 5. Neither 7.35.2 NMAC nor the Act uses the word.

didactic. The word does the work of an equivalency test in this document's scope and of an hour denomination in the practicum draft's. 7.35.3.10 NMAC Subsection A, page 4, keeps a program from another jurisdiction on the department list only while its curriculum stays equivalent to the "didactic educational requirements applicable for New Mexico certification", and removes it when "the didactic requirements of the educational program are no longer equivalent to those applicable for New Mexico certification". The module provisions of 7.35.3.18 NMAC, pages 11 to 12, state their minimums in didactic hours, and 7.35.3.19 NMAC Subsection A, page 12, opens the practicum to a student "after completing at least half of the didactic requirements and all of the simulated patient requirements of the educational requirements". No provision states which parts of a program are didactic and which are not. Neither 7.35.2 NMAC nor the Act uses the word.

adverse health event. The nearest the rule comes to content is the list of what a report of one must include. 7.35.3.20 NMAC Subsection L, pages 14 to 15, requires a healing center and a registrant of another approved location to "report any potential adverse health event associated with medical psilocybin services to the department no later than two business days" after becoming aware of it, and requires reports to include "Any challenging psychological, emotional, or behavioral reactions", "Any participant reaction requiring medical or therapeutic attention", "Any incident requiring emergency response", and "Any other incident which has a negative impact on a patient". A location's safety plan must include "adverse health event response and reporting", 7.35.3.11 NMAC Paragraph (15) of Subsection A, page 5; patient information must cover "How to report an adverse health event to the department", 7.35.3.20 NMAC Paragraph (5) of Subsection G, page 14; and a practitioner's administration records must note "Any adverse health events", 7.35.3.13 NMAC Subsection C, page 9. Whether the report-contents list is the definition is stated nowhere: it is a list of what reports include, not of what an event is. Neither 7.35.2 NMAC nor the Act uses the term.

administrative review committee. All four occurrences sit in 7.35.3.25 NMAC, page 16. "The administrative review shall be conducted by the administrative review committee.", Subsection C; "The decision of the administrative review committee is the final decision of the informal administrative review proceeding.", Paragraph (2) of Subsection D; and Subsection E provides that "Except as otherwise provided by law, there shall be no right to judicial review of a decision by the administrative review committee." Nothing in 7.35.3 NMAC or 7.35.2 NMAC constitutes the committee, states who sits on it, or says how it is appointed, and the Act does not use the term. The review note at 7.35.3.25 above records who must answer.

graduate. Four occurrences, and the fullest statement of what graduation takes is a list inside a subsection the Wilson working redline proposes to strike. An educational program must plan for enrolled students to "complete all required components and graduate within two years of enrollment", 7.35.3.12 NMAC Paragraph (18) of Subsection A, page 7; mentoring runs "after graduation and after practicum hours are completed", 7.35.3.17 NMAC Subsection A, page 10; a student using the test-out option completes "the New Mexico Module, certifications, simulated patient, and practicum requirements to graduate from the educational program", 7.35.3.17 NMAC Subsection B, page 11; and the practicum-hours waiver reaches an applicant who "Graduates from an educational program that the department certifies by December 31, 2027", 7.35.3.19 NMAC Paragraph (3) of Subsection G, page 13. Neither 7.35.2 NMAC nor the Act uses the word.

simulated patient. The rule requires the experience and never says what a simulated patient is. "A simulated patient experience of no less than 5 hours", 7.35.3.18 NMAC Subsection C, page 12; the practicum opens only after "all of the simulated patient requirements" are complete, 7.35.3.19 NMAC Subsection A, page 12; and the test-out option leaves the requirement standing, 7.35.3.17 NMAC Subsection B, page 11. Neither 7.35.2 NMAC nor the Act uses the term.

medical psilocybin services. Three occurrences, against a two-word term both other sources define. A facilitator "is authorized to work alongside a practitioner during medical psilocybin services", 7.35.3.13 NMAC Subsection B, page 8; the education requirements reach all certificants "who provide medical psilocybin services", 7.35.3.18 NMAC Subsection A, page 11; and the reporting duty covers "any potential adverse health event associated with medical psilocybin services", 7.35.3.20 NMAC Subsection L, pages 14 to 15. Section 3(D) of the Act and 7.35.2.7 NMAC define "Medical services", identically, as "services provided to a patient in an approved setting before, during and after the ingestion of psilocybin", including a preparation session, an administration session and an integration session. Whether the three-word term means the same as the defined two-word term is stated nowhere in this record.

equity and access fund. One occurrence, and no source names a fund by this name. A patient application must include "Information required for patient participation in the Equity and Access Fund (as applicable)", 7.35.3.8 NMAC Paragraph (7) of Subsection B, page 2. The Act creates two funds in Section 11, neither by this name: the "medical psilocybin treatment equity fund", to be used "to fund treatments of qualified patients who meet income requirements determined by rule of the department", and the "medical psilocybin research fund". 7.35.2 NMAC uses neither name. Whether the Equity and Access Fund is the Act's medical psilocybin treatment equity fund is stated nowhere in this record.

Addendum B Dependencies across the two drafting scopes

Provisions in these four documents that operate on, or are operated on by, a provision the practicum amendment draft in *amendments/* reaches. Nothing in these documents amends a provision in that draft's scope. Where a fix would require one, it is recorded here and left undrafted.

Provision here	Provision in the practicum draft's scope	Relationship	Handled
7.35.3.11 A(11), page 5	7.35.3.14 C, page 9	7.35.3.14 C conditions healing center owner and employee authority on being registered with the department, and no registration exists. The registration is drafted here, at the point where the healing center already files the names.	Drafted here. The practicum draft flagged 7.35.3.14 C and did not amend it, and it is not amended here either.
7.35.3.7, page 1	7.35.3.20 H(5), page 14	The definition of student drafted here is taken from the qualified student test in 7.35.3.20 H(5). The practicum draft amends that paragraph for the permit title and does not change the test.	Drafted here, sourced there. If the test changes, the definition follows it.
7.35.3.7, page 1	7.35.3.19 A, page 12	The definition of practicum drafted here is taken from 7.35.3.19 A, which provides that an individual seeking certification as a practitioner or facilitator "shall participate in supervised practice training, otherwise referred to as a" practicum. The practicum draft amends the hour figures in the same subsection and keeps that phrase.	Drafted here, sourced there. If the phrase changes, the definition follows it.
7.35.3.7, page 1	7.35.3.14 B, page 9	7.35.3.14 B authorizes facilitators to possess psilocybin products and provide them to qualified patients. Whether a facilitator is a clinician under Section 3(B) of the Medical Psilocybin Act, and so within the Section 5 exemption, governs whether that authority holds. This is the administering half of Paragraph (2) of Subsection B of Section 5 of the Act. The practicum draft reads the taking half of the same provision against Subsection H of Section 3, and its proposed 7.35.3.29 states the statutory amendment that half would need and conditions the section on it, so the two drafts read the same provision consistently.	Not drafted. Recorded at 7.35.3.7 as a question for department counsel.
7.35.3.7, page 1	7.35.3.14 A and C, page 9, with 7.35.3.20 D, page 14	The position of a student is not in doubt and is not raised in these documents. The practicum draft locates the provision of the medicine in the practitioner or the healing center, and 7.35.3.20 D provides that "students completing their practicums may be present during an administration session". The definition of student drafted here does not disturb that.	Settled in the practicum draft. Nothing here contradicts it.
7.35.3.13 B, page 8	7.35.3.14 B, 7.35.3.19 C, 7.35.3.20 H(5)	The facilitator scope of work in 7.35.3.13 B is narrower than the authority the other three provisions confer, and "direct supervision" is not defined anywhere in the rule.	Not drafted. Any amendment has to be coordinated with the practicum draft.
7.35.3.17 A, pages 10 to 11	Proposed 7.35.3.19 H, which exists only in that draft's re-lettering of 7.35.3.19	Whether the consultation requirement is stated in hours or in cases, and whether it sits at the proposed 7.35.3.19 H or at 7.35.3.17 A, is open in the practicum document for Dr. Metz, Ms. Wilson and Dr. Leeman. As published, 7.35.3.19 NMAC runs A through G and has no Subsection H.	Not drafted. 7.35.3.17 A is left exactly as published so that their answer governs.
7.35.3.10 D(1), page 5	7.35.3.19 G(4), page 13, which that draft re-letters as 7.35.3.19 K	Two waivers of the practicum hours requirement each set a 40-hour floor for applications received by December 31, 2027. For the same 40 hours of contact time, 7.35.3.10 D(1) requires "two separate group sessions" and 7.35.3.19 G(4) requires "A minimum of one group session". The practicum draft records the conflict at its 7.35.3.19 K and amends neither provision, on the ground that 7.35.3.10 is outside its scope.	Not drafted here either. Conforming one to the other is a choice between two figures already in the rule, so neither document makes it. The provision is reproduced at 7.35.3.10 above with the sharpened record, and the choice is put to the committee there.
7.35.3.16 A and C, page 10	7.35.3.18, pages 11 to 12	The third-party evaluation assesses the curriculum that 7.35.3.18 requires. Curing the evaluator conflict does not touch the curriculum, and the practicum draft's changes to the curriculum do not touch the conflict.	Drafted here. No coordination needed.

Provision here	Provision in the practicum draft's scope	Relationship	Handled
7.35.3.20 heading, page 13	7.35.3.20 heading, page 13	The heading reads 7.34.3.20. The practicum draft corrects it.	Not duplicated here. The correction stands in the practicum draft.

Addendum C Defects recorded and not drafted, and why

Every defect this record found in the twenty-four sections, that this draft does not amend. The reason in each row is the reason no language is proposed, not an assessment of severity. Each has a review note at the provision named, stating the choices and who decides.

Provision	Defect	Why nothing is drafted
7.35.3.5, page 1, with the history notes at page 19	Twenty-six history notes carry the placeholder "xx/xx/2026"; the notes for 7.35.3.27 and 7.35.3.28 carry 9/22/2026. Under 7.35.3.5 a later date cited at the end of a section controls, so two sections would take effect before the rest has a date.	No source supplies the intended effective date, and which way to conform is the department's to choose.
7.35.3.7, page 1	Ten of the sixteen undefined terms have no definition in any source, including healing center, 54 occurrences, and certifying clinician, 53 occurrences.	Drafting a defined term with regulatory consequence from no source would be composing it. Addendum A records the slate and assembles the material for each of the ten.
7.35.3.9 B, page 3	A renewal application is due no less than 30 days before expiry, and the department has no deadline to decide it, so a certification can lapse while a timely renewal is pending. Nothing continues the certification in the interim.	The fix is a continued-validity provision, which is a new grant, and no source in this record proposes its terms.
7.35.3.9 E(1) and F, page 3	A practitioner applicant must document a professional license "(e.g. PSY, LSW, LCSW)" with no stated scope of practice, and a facilitator applicant needs no license at all. The Wilson working redline comments "Also no scope here - what happened to therapy scope? It's missing - if DOH wants that, they need to include it." and "without specifying, they're setting themselves up to have athletic trainers and massage therapists apply and then not have a regulation to point to explain why that's not sufficient."	The redline raises the gap and supplies no list of qualifying licenses, and no other source in this record supplies one.
7.35.3.8 C, page 2, with 7.35.3.9 A(1), page 2	The department has 30 business days to decide a patient application and 30 calendar days to decide a certificant application.	Conforming one to the other is a choice between two figures already in the rule, and both bind the department.
7.35.3.10 A(2) and A(3), pages 4 to 5	The list of out-of-jurisdiction programs populates only when an individual application relying on a program is approved, so it is empty at launch. A(3) refers to programs approved by Oregon and Colorado "as of December 31, 2027", which is after the December 31, 2027 waiver in D(1) has closed.	Both fixes are new grants of authority to the department, and no source in this record proposes their terms.
7.35.3.10 D(1), page 5, with 7.35.3.19 G(4), page 13	The two 40-hour waivers set different group-session minimums, two against one.	Conforming one to the other is a choice between two figures already in the rule. The practicum draft records the same conflict and amends neither provision, so neither document makes the choice. Addendum B.
7.35.3.11 A(22)(d) and (e), and B(10) (d), pages 5 to 7	The 15-minute threshold for a remote natural environment. The Wilson working redline asks twice whether it should be 30.	A question in the source, not a recommendation, and no source supplies a figure.
7.35.3.11, page 5	The two-person universal requirement the Wilson working redline adds, which its author records did not reach the published rule.	The author records that she does not know where the provision belongs. Placing it would answer her question.
7.35.3.12 A(20) and B(1), pages 7 to 8	An application filed on or before December 31, 2027 may be approved without the third-party evaluation provided the evaluation is submitted by December 31, 2027, so the deferral shortens to nothing as that date approaches and is unavailable on the last day.	The fix is a period, and no source in this record supplies one. Tying it to the certification term would be a choice about how long a program may operate unevaluated.
7.35.3.12, pages 7 to 8, against the July 9, 2026 draft	The grounds for denying an educational program application present in the July 9 draft do not appear in the published rule. Finding M9.	Restoring deleted grounds is a substantive policy decision, and this record cannot show whether the deletion was deliberate.

Provision	Defect	Why nothing is drafted
7.35.3.17 A, pages 10 to 11	The mentoring obligation, and whether it is stated in hours or cases and where it sits.	Open in the practicum document for three named people. Addendum B.
7.35.3.17 B, page 11	The test-out price cap is a fraction of the price of "each of the educational modules", which assumes per-module pricing. The Wilson working redline strikes the subsection.	Whether the option survives at all is the committee's decision, and a restated cap is worth drafting only if it does.
7.35.3.20 A, page 13, with 7.35.3.21 A, page 15	Healing centers must keep a roster of all qualified patients intended to use and who have used the location, and the department may interview patients.	Whether the roster should be kept in a form that does not identify patients turns on the department's data needs, which no source in this record states.
7.35.3.20, page 13	The term "registrant of another approved location", 10 occurrences, against the certification that 7.35.3.11 B issues.	The definition drafted at 7.35.3.7 ties the two words together. Conforming all 10 occurrences instead is a drafting choice, and this document does not make it.
7.35.3.21, page 15	No interval is set for department assessments, and none for repeating a third-party evaluation after the first.	An interval is a figure and a demand on department capacity. No source supplies either.
7.35.3.25 C and D(2), page 16	The administrative review committee decides a denied patient application and is constituted nowhere in 7.35.3 NMAC or 7.35.2 NMAC. Four occurrences, all in this section.	Only the department can say who the committee is.
7.35.3.25 E, page 16	"Except as otherwise provided by law, there shall be no right to judicial review of a decision by the administrative review committee." The Wilson working redline comments on it.	Whether the provision is within the department's authority is a legal question for department counsel.
7.35.3.24, page 15	The July 9, 2026 draft allowed a complaint from patients or staff; the published section allows one from a qualified patient or certificant, so staff who are neither have no route.	Restoring standing for staff is a policy decision. The confidentiality assurance the July 9 draft carried is restored in the redline above, because that text exists in a source.
7.35.3.27, pages 16 to 19	A certificant suspended immediately under Subsection A can be out of practice more than 135 days before a final decision, and there is no expedited track. Finding N5.	An expedited schedule is a set of periods binding the department and the secretary, and no source supplies them.
7.35.3.8 D, 7.35.3.25 A, 7.35.3.27 C(7), pages 2, 16 and 17	Three provisions describe review of a denial and their scopes overlap, so a patient applicant denied on the merits may have both an informal review and a hearing. Finding M23.	The redline fixes the name of the proceeding only. Which track applies to which denial is a question of what process is owed.

Out of scope by decision. The controlled-substance number requirement for certifying clinicians. It appears at Paragraph (3) of Subsection B of 7.35.3.8 NMAC, page 1, and at Paragraph (2) of Subsection D of 7.35.3.9 NMAC, page 3. Paragraph (2) is reproduced in the redline above because the renumbering of the paragraphs around it required it. It is reproduced as published and is not amended, and nothing is proposed about it.

Addendum D Mechanical defects across all twenty-eight sections

Numbering, dates and drafting form, checked across the whole of 7.35.3 NMAC rather than only the sections these documents amend. Every count and every entry is produced from the text layer of the published rule each time this document is built.

Item	Count	Where	Corrected
Section headings reading 7.34.3 instead of 7.35.3	5 of 28	7.34.3.13, 7.34.3.14, 7.34.3.20, 7.34.3.23, 7.34.3.25	7.35.3.13 in the document 4 above; 7.35.3.14 in the practicum draft; 7.35.3.20 in the practicum draft; 7.35.3.23 in the document 4 above; 7.35.3.25 in the document 4 above
History notes carrying a real effective date rather than a placeholder	2 of 28	7.35.3.27 reads 9/22/2026, 7.35.3.28 reads 9/22/2026	Not corrected. Recorded at 7.35.3.5 above.
History notes omitting the "- N" new-section designator	9 of 28	7.35.3.11, 7.35.3.13, 7.35.3.14, 7.35.3.20, 7.35.3.22, 7.35.3.23, 7.35.3.24, 7.35.3.25, 7.35.3.26	Not corrected. Mechanical, and the designator is the department's to set.
Numbering gap inside a subsection	1	7.35.3.9 D runs (1), (2), (4), (5), (6). There is no (3).	Renumbered in document 4 above.
Subsections lettered "(A)" rather than "A."	4	7.35.3.11 A, and 7.35.3.14 A, B and C. Letters found: A, B, C	7.35.3.11 A in document 3 above. 7.35.3.14 in the practicum draft.
Sentence with no verb governing the decision	1	7.35.3.25 D(1), page 16. Carried over from the July 9, 2026 draft.	Corrected in document 4 above.
Sentence that does not complete	1	7.35.3.18 F, page 12.	In the practicum draft's scope. Corrected there.
Presiding official named inconsistently	1	7.35.3.27 M reads "hearing examiner"; the rest of 7.35.3.27 reads "hearing officer".	Corrected in document 4 above.
Duplicated words	1	7.35.3.24 reads "the electronic system designated by the department electronic system".	Corrected in document 4 above.
Spacing inside a heading	1	7.35.3.16 C reads "Conflict of -interest prohibitions". The July 9, 2026 draft read "Conflict-of-interest prohibitions".	Corrected in document 2 above.
Class named in the lead-in but not in the operative words	1	7.35.3.27 B(8) opens "for certifying clinicians and practitioners" and then refers only to "the clinician".	Corrected in document 4 above.
Patient described as certified rather than enrolled	1	7.35.3.27 C(1) reads "a certified patient".	Corrected in document 4 above.

Addendum E The questions, one line each, grouped by who decides

The twenty-four review notes in this document each state an issue, the choices, and who decides. This page re-presents them as questions, one line each, grouped by who decides. It introduces nothing new: the note at the provision named in each row is the controlling statement, and where one note puts different questions to different decision-makers it appears once per question.

AT	WHAT IS BEING ASKED
Department of Health staff	
7.35.3.5	Which effective date fills the placeholders, and do 7.35.3.27 and 7.35.3.28 keep September 22, 2026?
7.35.3.9 D	Is the gap at paragraph (3) a numbering error, or the trace of a requirement that was meant to stay?
7.35.3.10 C	Is the six months after a first denial a waiting period or a deadline to re-apply, in all four places the sentence appears?
7.35.3.11 A(11)	How does the owner and employee registration stay current between application and the two-year renewal?
7.35.3.11 D	Is a 90-day term with renewal workable, what does a renewal contain, and what period binds the department to decide one?
7.35.3.15 A	What period binds the department to act on a reported program change, and do the 60 days drafted by analogy stand?
7.35.3.16 B(2)	Was the change from or to and between the July 9 and July 23 drafts deliberate, and should the July 9 reading return?
7.35.3.20 heading and A	Does the definition drafted at 7.35.3.7 settle registrant, or should all 10 occurrences here read certificant?
7.35.3.21 A	How often does the department assess, and how often must a third-party evaluation be repeated?
Department of Health staff with department counsel	
7.35.3.3	Should the department confirm the codification of the Medical Psilocybin Act before the rule is filed?
7.35.3.7	Is a facilitator within Section 3(B) of the Act, and so within the Section 5 exemption, given the authority 7.35.3.14 NMAC confers?
7.35.3.8 D	Are the informal review and the hearing alternatives, and which denials reach which track?
7.35.3.24	Does staff standing to complain return, and how far does the restored confidentiality assurance reach?
7.35.3.25 D(1)	Who sits on the administrative review committee, and is the bar on judicial review within the department's authority?
7.35.3.27 M	Does a hearing that follows an immediate suspension get an expedited schedule?
The Training and Education Committee	
7.35.3.2	Does the scope keep clinician as published, or conform to the vocabulary the rest of the part uses?
7.35.3.7	Which of the ten undefined terms go to the department for definition, healing center and certifying clinician first?
7.35.3.10 A(2)	Should the department be asked for authority to place a program on the list on its own initiative?
7.35.3.10 D(1)	Which group-session minimum stands in the 40-hour waiver, the two here or the one of 7.35.3.19 G(4)?
7.35.3.13 B	What does direct supervision mean, and is it defined in this part or the three wider provisions narrowed?
7.35.3.16 heading and A	Does the engagement-fee cure go forward at Subsection A, at the conflict rule, or by the department engaging the team?
7.35.3.16 B(2)	Can a three-person team cover all three domains at three years each, or should a lower standard be requested?
7.35.3.17 B	Does the test-out option survive, and if it does, how is its price cap restated for a single-price program?
The board	
7.35.3.11 B(9)	Does the owner notice stay for a patient's own residence, or does the access exemption drafted here stand?
7.35.3.20 heading and A	Should the location's patient roster be kept in a form that does not identify patients to the department?
7.35.3.21 A	Should a patient interview outside a session or the patient's home require the patient's consent?
Dr. Metz, Ms. Wilson and Dr. Leeman	
7.35.3.17 A	Is the consultation requirement stated in hours or in cases, and does it sit at the proposed 7.35.3.19 H or here?
7.35.3.11 B(10)	Where does the two-person requirement belong, and does the 15-minute threshold become 30?

Sources. Rule as published: *docs/documents/rules-draft-2026-07-23-published.pdf*, 19 pages, 7.35.3.1 through 7.35.3.28. Prior board-meeting draft, used for new against carried-over determinations: *docs/documents/rules-draft-2026-07-09.pdf*. Defined terms: 7.35.2 NMAC as adopted effective June 23, 2026, at *source-text/7.35.2-NMAC-adopted-2026-06-23.txt*. Statute: the Medical Psilocybin Act, Senate Bill 219, 57th Legislature, First Session, 2025, as passed; Section 1 provides that Sections 1 through 11 of the act may be cited as the Medical Psilocybin Act, and those section numbers are the ones cited here. This record does not verify any NMSA 1978 section number. Working redline: Denali Wilson, NMAC 7.35.3, July 25, 2026, tracked changes and comments as exported from the document file. July 17, 2026 transcripts, morning Advisory Board and afternoon Training and Education Committee, both labeled "UNOFFICIAL AUTO-GENERATED TRANSCRIPT. NO SPEAKER ATTRIBUTION."; no speaker is named from either transcript in this document. Inventory relied on and re-verified: *analysis/july23-rule-concerns.md*.

Method. The left column reproduces the rule as published, verbatim, with line breaks introduced by the PDF collapsed to single spaces and hyphenation introduced by a line break rejoined. Nothing else is altered. Every block was checked against the text layer of the published rule by exact contiguous match, and the build aborts if any block fails. *amendments-remainder/audit.py* re-checks every verifiable claim in these documents and exits non-zero on any failure; *amendments-remainder/AUDIT.md* is its output.

Status. Working draft v2. Not a filing, not submitted, and not adopted rule text. Nothing in *amendments/* or *docs/* was modified by this work.